
Roche

This presentation contains certain forward-looking statements. These forward-looking statements may be identified by words such as ‘believes’, ‘expects’, ‘anticipates’, ‘projects’, ‘intends’, ‘should’, ‘seeks’, ‘estimates’, ‘future’ or similar expressions or by discussion of, among other things, strategy, goals, plans or intentions. Various factors may cause actual results to differ materially in the future from those reflected in forward-looking statements contained in this presentation, among others:

1 pricing and product initiatives of competitors;
2 legislative and regulatory developments and economic conditions;
3 delay or inability in obtaining regulatory approvals or bringing products to market;
4 fluctuations in currency exchange rates and general financial market conditions;
5 uncertainties in the discovery, development or marketing of new products or new uses of existing products, including without limitation negative results of clinical trials or research projects, unexpected side-effects of pipeline or marketed products;
6 increased government pricing pressures;
7 interruptions in production;
8 loss of or inability to obtain adequate protection for intellectual property rights;
9 litigation;
10 loss of key executives or other employees; and
11 adverse publicity and news coverage.

Any statements regarding earnings per share growth is not a profit forecast and should not be interpreted to mean that Roche’s earnings or earnings per share for this year or any subsequent period will necessarily match or exceed the historical published earnings or earnings per share of Roche.

For marketed products discussed in this presentation, please see full prescribing information on our website www.roche.com

All mentioned trademarks are legally protected.

Roche

2025 results

Basel, 29 January 2026



Group

Thomas Schinecker
Chief Executive Officer

FY 2025 performance

Delivering the next innovation cycle

Outlook

2025: Strong financial and pipeline performance

FY results at CER

- Group sales growth +7%, +9% for Pharma and +2% for Diagnostics (despite China healthcare pricing reforms)
- Strong operating performance with Core OP +13% and Core OP margin +1.9%p; Core EPS +11%
- Full year LOE impact was CHF -0.7bn

Key milestones achieved in Q4 25 / YTD 26

- Pharma regulatory: EU approval Gazyva in LN, US and EU approval Lunsumio SC in 3L+ FL, US filing giredestrant in post-CDKi ER+/HER2- mBC
- Pharma readouts: Positive Ph III (FENtrepid, FENhance 2) fenebrutinib in PPMS and RMS, positive Ph III (METEOROID) Enspryng in MOG-AD, positive Ph III (lidERA) giredestrant in adjuvant ER+/HER2- BC, positive Ph III (COMMUTE) PiaSky in aHUS, positive Ph III (INShore, ALLEGORY) Gazyva in INS and SLE; positive Ph II (103) CT-388 in obesity
- Diagnostics regulatory: Elecsys Dengue Ag CE mark, cobas BV/CV CE mark, cobas i601 Mass Spectrometry wave 1 ipacks CE mark, navify Digital Pathology 3.0 CE mark

Significant newsflow in 2026 ahead

- Pivotal Ph III readouts: fenebrutinib (FENhance 1) in RMS, giredestrant in 1L ER+/HER2- mBC, Itovebi in 1L *PIK3CA*-mut HER2+ mBC and post CDKi HR+/HER2- mBC, divarasib in 2L KRASG12C+ NSCLC, Lunsumio in 2L+ FL, Gazyva in MN, sefaxersen in IgAN
- Ph II readouts: emugrobart in FSHD*, SMA* and obesity, CT-388 in obesity with T2D (104), CT-996 in obesity, CT-868 in T1D with obesity*, petrelintide in obesity
- Diagnostic launches: AXELIOS Sequencing Solution, Elecsys pTau217, Elecsys NfL, Elecsys IGRA TB, cobas MPX-E, cobas HDV, eplex gastrointestinal panel, VENTANA BAP1

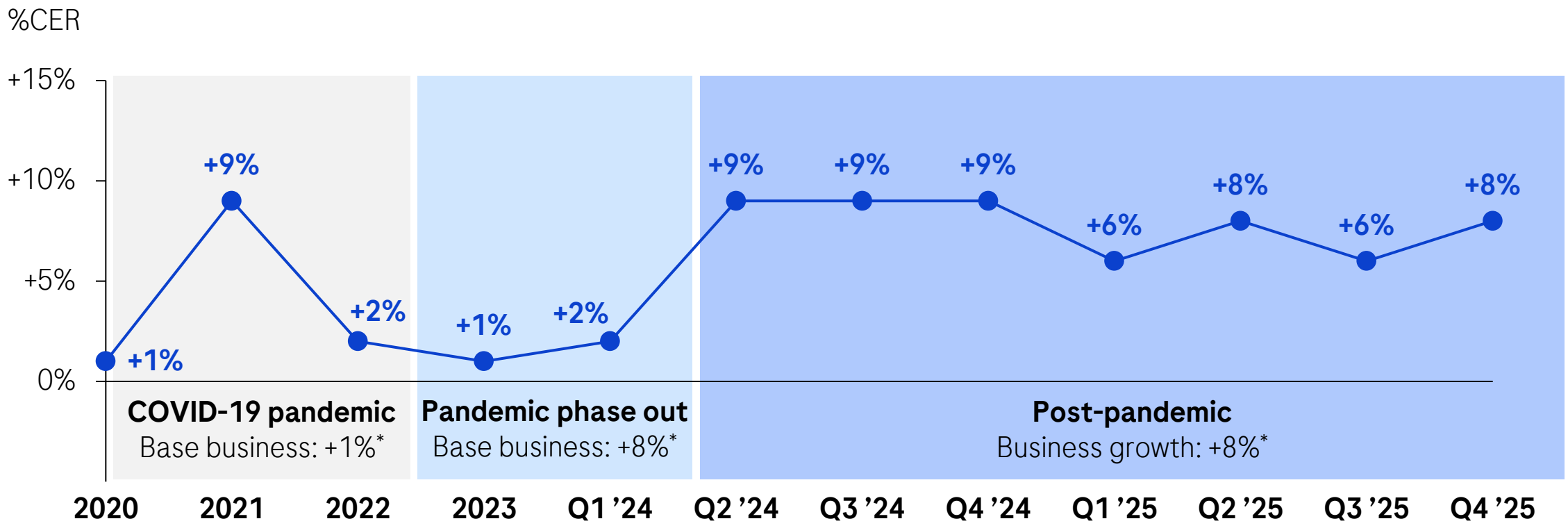
* Data inhouse, to be presented

aHUS: Atypical hemolytic uremic syndrome; BV/CV: Bacterial vaginosis/candida vaginitis; CDKi: Cyclin-dependent kinase inhibitor; CER: Constant exchange rates (avg. full year 2024); ER: Estrogen receptor; FL: Follicular lymphoma; FSHD: Facioscapulohumeral muscular dystrophy; HER2: Human epidermal growth factor; HR: Hormone receptor; IgAN: IgA nephropathy; INS: idiopathic nephrotic syndrome; LN: Lupus nephritis; LOE: Loss of exclusivity incl. global losses of Avastin, Herceptin, MabThera/Rituxan, Esbriet, Lucentis and Actemra (m)BC: (Metastatic) breast cancer; MN: Membranous nephropathy; MOG-AD: Myelin oligodendrocyte glycoprotein antibody disease; NSCLC: Non-small cell lung cancer; PPMS: Primary progressive multiple sclerosis; RMS: Relapsing multiple sclerosis; SC: subcutaneous; SLE: Systemic lupus erythematosus; SMA: Spinal muscular atrophy; T1D: Type-1 diabetes

2025: Consistent strong growth

Diagnostics impacted by healthcare pricing reforms in China in 2025

Group sales growth



All growth rates at CER: Constant exchange rates (avg. full year of respective years); Base business: Pharma excluding Ronapreve and Diagnostics excluding COVID-19 diagnostic tests; * Average growth rate of quarterly CER growth rates for specified period

2025: Continued strong Pharma sales driving Group growth

Diagnostics excluding China grew +7% at CER

	FY 2025 CHFbn	FY 2024 CHFbn	Change in % CHF	Change in % CER
Pharmaceuticals Division	**47.7**	46.2	**3**	**9**
Diagnostics Division	**13.8**	14.3	**-3**	**2**
Roche Group	**61.5**	60.5	**2**	**7**

CER: Constant exchange rates (avg. full year 2024)

Key growth drivers of the Roche portfolio

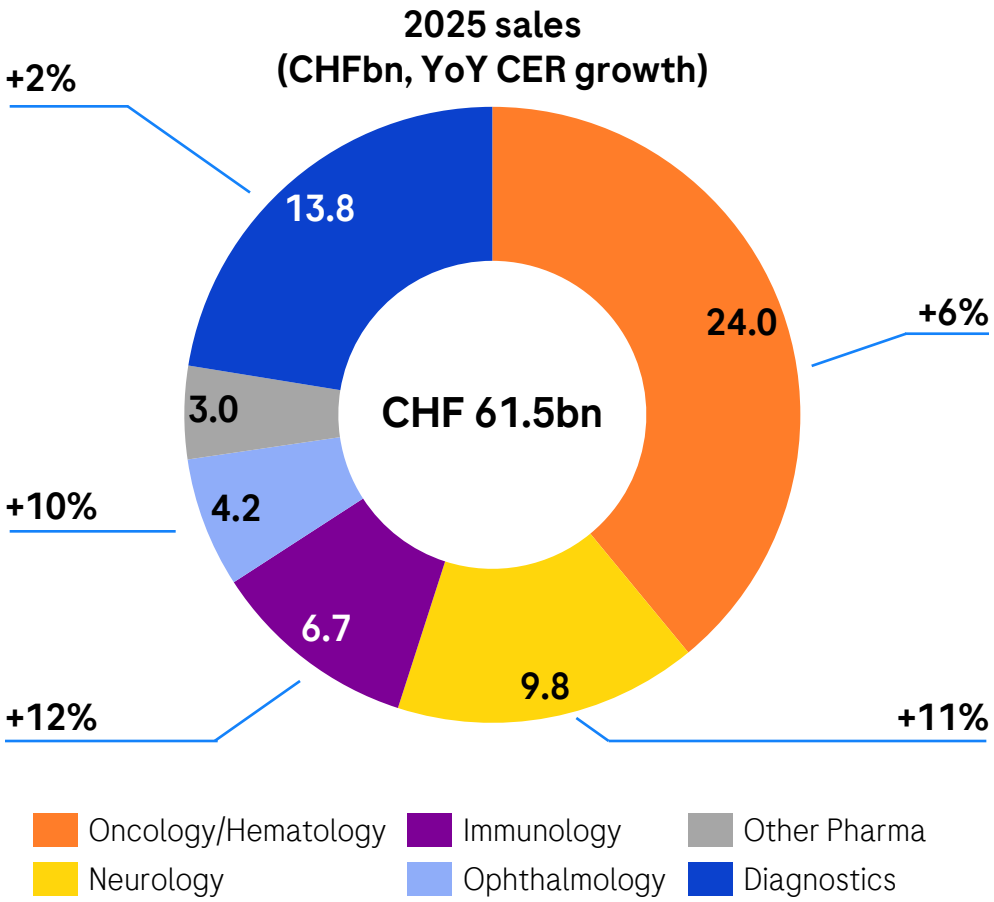
All therapeutic areas delivering strong growth

Diagnostics: Molecular Lab and Pathology Lab with strong growth; Core Lab impacted by China healthcare pricing reforms; excluding China business grew +7% at CER

Vabysmo: Continued strong global growth; US market recovery expected in 2026

Xolair: Strong uptake in food allergy, biosimilar launch expected H2 2026

Gazyva: LN launch in US and EU ongoing



Phesgo: Global conversion rates >50%*; HER2+ BC franchise to peak in 2026 with strong tail

Tecentriq: Return to low single digit growth

Alecensa: Growth driven by US and Japan

Polivy: Strong 1L DLBCL uptake; US patient share reaching 36%

Columvi / Lunsumio: Driven by 2L+ launch and 3L+ DLBCL; strong 3L+ FL launch

Hemlibra: Continued global growth

Ocrevus: >50% of global Q4 growth driven by SC

Evrysdi: Leading in SMA with >21k pts treated

*Perjeta/Phesgo conversion rate calculated using volumes, currently taking 80 launch countries into account (78 countries at Q3 2025); BC: Breast cancer; CER: Constant exchange rates (avg. full year 2024); DLBCL: Diffuse large B-Cell lymphoma; FL: Follicular lymphoma; HER2: Human epidermal growth factor receptor; LN: Lupus nephritis; SC: Subcutaneous; SMA: Spinal muscular atrophy

Achieved 2025 upgraded guidance

LOE impact of -0.7bn

	Guidance	Results
Group sales growth[1]	Mid single digit sales growth	**+7%** ✓
Core EPS growth[1]	High single digit to low double digit Core EPS growth (from high single digit growth)	**+11%** ✓
Dividend outlook	Further increase dividend in Swiss francs[2]	**CHF 9.80** ✓

1. At CER: Constant exchange rates (avg. full year 2024); 2. 2025 dividend as proposed by the Board of Directors; LOE: Loss of exclusivity includes global losses of Avastin, Herceptin, MabThera/Rituxan, Esbriet, Lucentis and Actemra

Substantial 2025 pipeline progress across all TAs

12 positive Phase III readouts unlocking significant upside potential

Positive key Phase III readouts

giredestrant
- lidERA in adjuvant ER+ BC ✓
- evERA in 2L+ ER+ mBC ✓

fenebrutinib
- FENhance 2 in RMS ✓
- FENtrepid in PPMS ✓

GAZYVA
- ALLEGORY in SLE ✓
- INShore in INS ✓

vamikibart Enspryng
- SANDCAT/MEERKAT in UME ✗/✓
- SatraGO-1/SatraGO-2 in TED ✗/✓

Data support filing

10 NMEs new to Phase III

- **NXT007** in hemophilia A
- **cevostamab** in R/R MM
- **HER2 TKI** in HER2+ BC
- **trontinemab** in AD
- **prasinezumab** in PD
- **zosurabalpin** in bacterial infections
- **zilebesiran** in hypertension
- **CT-388** in obesity
- **CT-868** in T1D
- **pegozafermin** in MASH

Oncology/Hematology *Neurology* *Immunology* *Ophthalmology* *CVRM*

AD: Alzheimer's disease; CVRM: Cardiovascular, renal and metabolism; ER: Estrogen receptor; HER2: Human epidermal growth factor receptor; INS: Idiopathic nephrotic syndrome; MASH: Metabolic dysfunction-associated steatohepatitis; (m)BC: (Metastatic) breast cancer; MM: Multiple myeloma; NME: New molecular entity; PD: Parkinson's disease; PPMS: Primary progressive multiple sclerosis; RMS: Relapsing multiple sclerosis; R/R: Relapsing/remitting; SLE: Systemic lupus erythematosus; T1D/T2D: Type-1/2 diabetes; TED: Thyroid eye disease; TKI: Tyrosine kinase inhibitor; UME: Uveitic macular edema

Agreement with the US Government reached

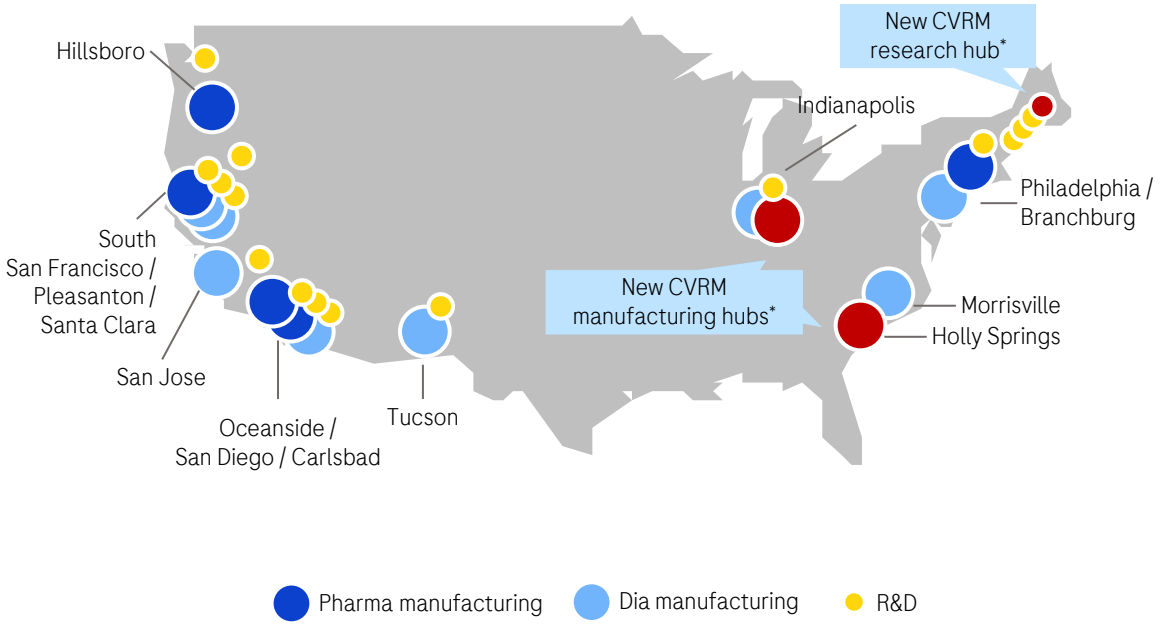
Roche portfolio exempt from tariffs and demo projects (GUARD and GLOBE)

Implications of the agreement

1. **Medicaid rebates**
   Genentech to offer states additional rebates comparable to prices in other wealthy countries
2. **Encouragement for other wealthy nations to reward biopharmaceutical innovation**
3. **Direct-to-patient medication access**
   Influenza portfolio made available in time for the flu season
4. **Investing in the United States**
   USD 50bn commitment into R&D and PP&E by 2029

USD 50bn investment in manufacturing, infrastructure, R&D

13+1* manufacturing and 15+1* R&D sites in the US



*One new Pharma CVRM manufacturing site (Holly Springs, groundbreaking in August 2025), one new Dia CGM manufacturing expansion (Indianapolis, to be constructed) and one CVRM focused R&D center (Boston, to be constructed); CVRM: Cardiovascular, renal and metabolism; PP&E: Property, plant and equipment

FY 2025 performance

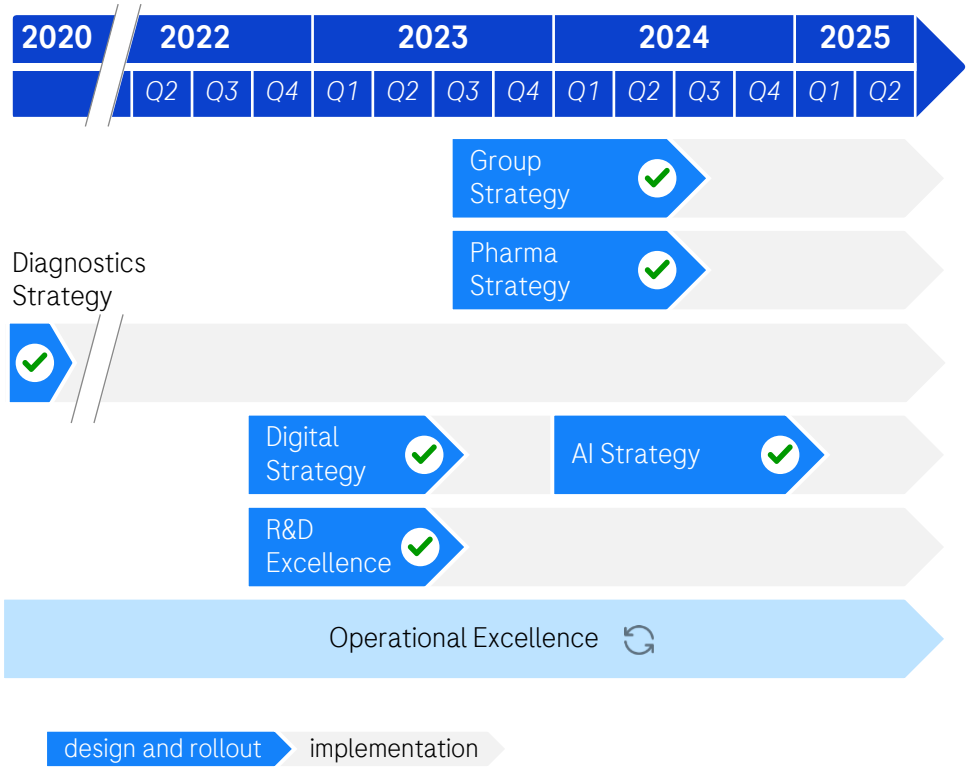
Delivering the next innovation cycle

Outlook

Substantial progress made across key initiatives

R&D Excellence efforts have already translated into a material shift of the portfolio value

Key initiatives



Key milestones achieved

- Introduction of the Bar
- Intentional pipeline focus with 5 priority TAs and E2E DAs
- Expanding into new Dia platform technologies
- AI implemented along the value chain with a focus on drug development
- High performing organization

R&D Excellence progress

>60%	of NMEs are "post Bar"[1]
66%	of late-stage projects with BID potential[2]
+60%	avg. peak sales per pipeline project[2,3]
+45%	total portfolio value[2,3]

1. "Post Bar" defined as NMEs entering the portfolio after YE2023 or advancing to the next clinical phase after YE2023 (including PivGo decisions); 2. Internal source; BID: Best in disease; DA: Disease area; E2E: End-to-end; NME: New molecular entity

Strong on-market portfolio with no patent cliff ahead

Significant upside potential being unlocked in 2025/26; key Ph III readouts 2027 onwards

Today's on-market Pharma portfolio to deliver growth until 2028

VABYSMO
POLIVY polatuzumab vedotin
Evrysdi risdiplam
TECENTRIQ atezolizumab
PHESGO pertuzumab/trastuzumab/hyaluronidase-zzxf SUBCUTANEOUS INJECTION
HEMLIBRA emicizumab-kxwh injection for subcutaneous use
COLUMVI glofitamab-gxbm
OCREVUS ocrelizumab
Lunsumio mosunetuzumab

Positive key Ph III readouts in 2025 enabling filings, including 3 NMEs

- **giredestrant** in ER+ BC
- **fenebrutinib** in RMS, PPMS
- **Gazyva** in SLE, INS
- **vamikibart** in UME
- **Enspryng** in TED

Next key NMEs expected to read out in 2027/28

- **afimkibart** in UC, CD
- **pegozafermin** in MASH
- **CT-388** in obesity
- **trontinemab** in AD
- **NXT007** in hemophilia A

Fast tracked

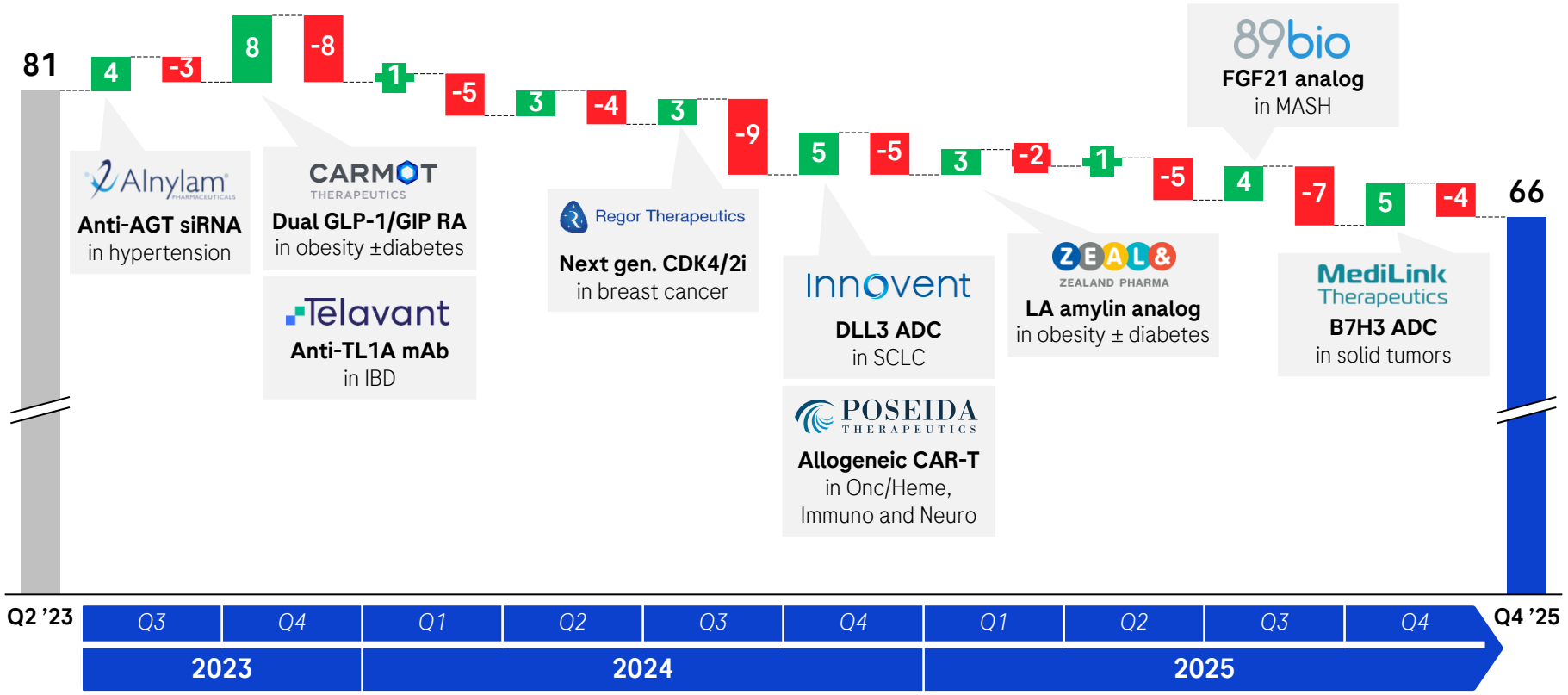
Pre-2025 launches | **2025 - 2026** | **2027+**

AD: Alzheimer's disease; BC: Breast cancer; CD: Crohn's disease; ER: Estrogen receptor; INS: idiopathic nephrotic syndrome; MASH: Metabolic dysfunction-associated steatohepatitis; NME: New molecular entity; PPMS: Primary progressive multiple sclerosis; RMS: Relapsing multiple sclerosis; SLE: Systemic lupus erythematosus; TED: Thyroid eye disease; UC: Ulcerative colitis; UME: Uveitic macular edema

Substantial progress on pipeline rejuvenation

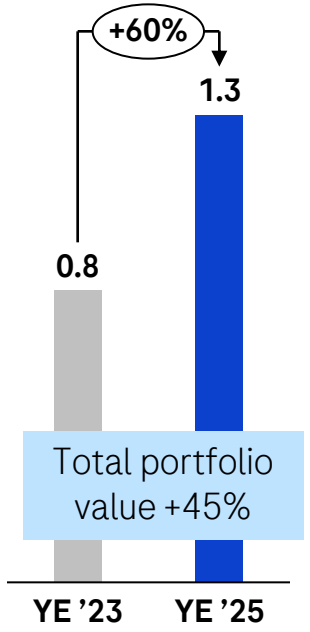
Prioritization of high-impact projects and strategic business development

Portfolio evolution (NME additions / removals)



Portfolio value

Average peak sales per pipeline project, CHF bn[1]



1. Source: internal data; ADC: Antibody-drug conjugate; AGT: Angiotensinogen; CAR-T: Chimeric antigen receptor T-cell; CDK4/2i: Cyclin dependent kinase-4/2 inhibitor; DLL3: Delta-like ligand 3; FGF21: Fibroblast growth factor 21; GIP: Glucose-dependent insulinotropic polypeptide; GLP-1: Glucagon-like peptide-1; IBD: inflammatory bowel disease; LA: Long acting; mAb: Monoclonal antibody; MASH: Metabolic dysfunction-associated steatohepatitis; NME: New molecular entity; RA: Receptor agonist; SCLC: Small-cell lung cancer; siRNA: Small interfering RNA; TL1A: Tumor necrosis factor-like cytokine 1A; Note: Chart Includes all assets from Ph I to Registration

Roche Diagnostics: Key technology launches driving future growth

CGM, Mass Spec and SBX each represent >1bn CHF opportunities

Accu-Chek® SmartGuide
(2024)



- 14-day real-time glucose sensor with predictive algorithms for 2 hours and night-time hypoglycemia

cobas® i601 Mass Spec
(2024)



- First fully integrated IVD platform for clinical mass spectrometry
- Launched in CE markets (incl. wave 1 menu), full US launch expected in 2026

AXELIOS (SBX)
(2026)



AXELIOS

- World record for fastest DNA sequencing, enabling human WGS in <4h*
- ASHG 2025 update: Bulk RNA sequencing, methylation mapping, multiomics integration, spatial analyses and growing network of early collaborators

*from a DNA sample to a final variant call file (VCF); IVD: In vitro diagnostics; SBX: Sequencing by expansion; WGS: Whole genome sequencing

FY 2025 performance

Delivering the next innovation cycle

Outlook

Sustaining pipeline momentum through 2026

10 key Ph III readouts; significant progress in obesity portfolio expected

Key regulatory milestones

- **giredestrant** filing in adjuvant ER+/HER2- eBC
- **giredestrant** approval in post CDKi ER+/HER2- mBC
- **Lunsumio + Polivy** approval in 2L+ DLBCL
- **Gazyva** filing in SLE, INS
- **Enspryng** US filing in TED
- **vamikibart** filing in UME

Key Phase III readouts

- **giredestrant** in 1L ER+/HER2- mBC
- **Itovebi** in post-CDKi *PIK3CA*-mut HR+ mBC
- **Itovebi** in 1L *PIK3CA*-mut HER2+ mBC
- **divarasib** in 2L KRASG12C+ NSCLC
- **Lunsumio** in 2L+ FL
- **fenebrutinib** in RMS
- **Enspryng** in MOG-AD ✓
- **Gazyva** in MN
- **sefaxersen*** in IgAN
- **Vabysmo** in CNV

Phase II readouts in obesity

- **CT-388** (Ph III to be initiated) ✓
- **CT-868** (Ph III to be initiated) ✓
- **CT-996**
- **petrelintide****
- **emugrobart**

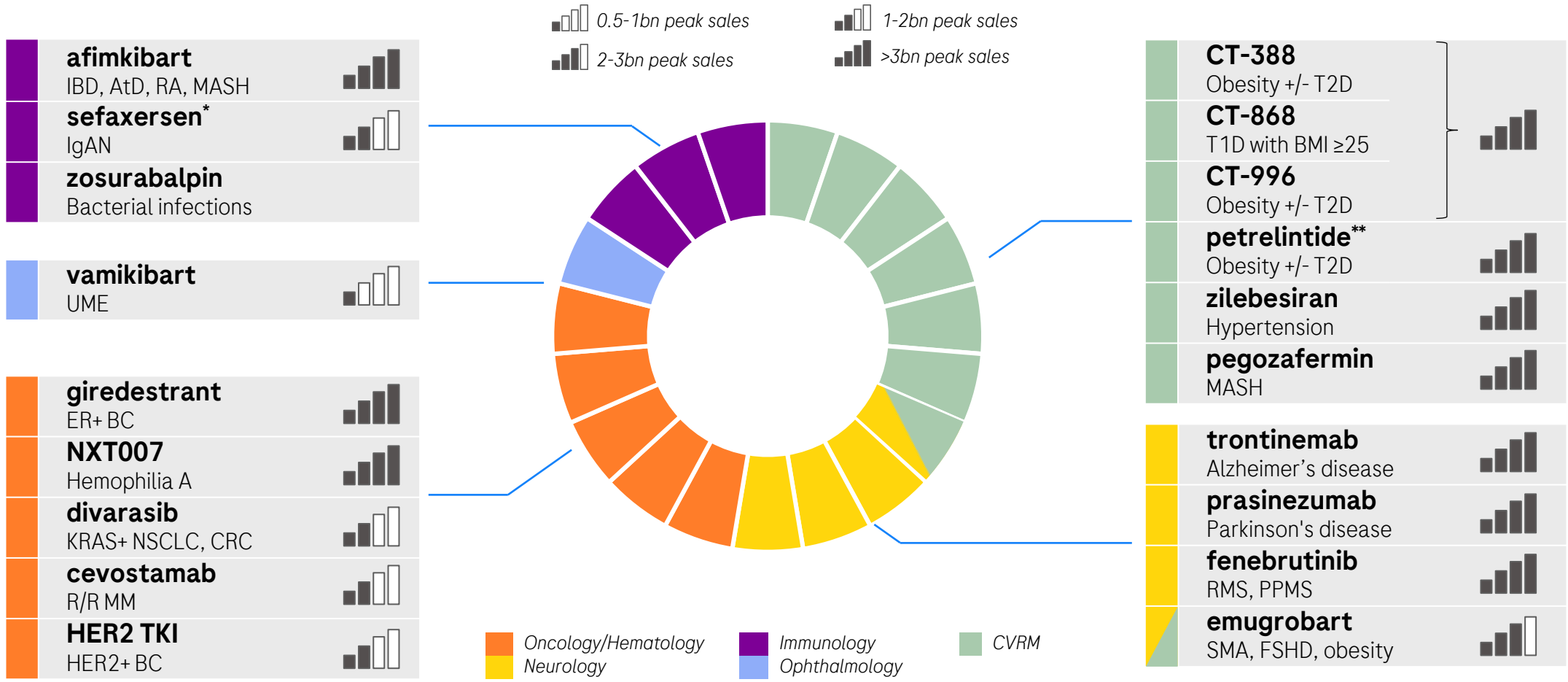
Oncology/Hematology *Neurology* *Immunology* *Ophthalmology* *CVRM*

✓ *Positive data*

*In collaboration with Ionis; **In collaboration with Zealand Pharma; CDKi: Cyclin-dependent kinase inhibitor; CNV: Choroidal neovascularization; CVRM: Cardiovascular, renal and metabolism; DLBCL: Diffuse large B-Cell lymphoma; (e)BC: (Early) breast cancer; ER: Estrogen receptor; FL: Follicular lymphoma; HR: Hormone receptor; HER2: Human epidermal growth factor receptor; IgAN: IgA nephropathy; INS: idiopathic nephrotic syndrome; (m)BC: (Metastatic) breast cancer; MN: Membranous nephropathy; MOG-AD: Myelin oligodendrocyte glycoprotein antibody disease; NSCLC: Non-small cell lung cancer; RMS: Relapsing multiple sclerosis; SLE: Systemic lupus erythematosus; TED: Thyroid eye disease; UME: Uveitic macular edema

Future growth outlook: Up to 19 NMEs with launch potential by 2030

First post Bar NMEs expected to readout in 2027; peak sales assumptions for fenebrutinib and sefaxersen raised



Peak sales shown unadjusted; post Bar NMEs includes: afimkibart, NXT007, cevostamab, CT-388, CT-868, CT-996, petrelintide, zilebesiran, pegozafermin, trontinemab, prasinezumab; *In collaboration with Ionis; **Zealand Pharma and Roche entered collaboration in 2025; CVRM: Cardiovascular, renal and metabolism; NME: New molecular entity

2026 guidance

LOE impact of roughly CHF 1bn expected for 2026

Group sales growth[1]	Mid single digit sales growth
Core EPS growth[1]	High single digit Core EPS growth
Dividend outlook	Further increase dividend in Swiss francs

1. At CER: Constant exchange rates (avg. full year 2025); LOE: Loss of exclusivity includes global losses of Avastin, Herceptin, MabThera/Rituxan, Lucentis and Actemra



Finance

Alan Hippe
Chief Financial Officer

Results

Cash & balance sheet

Currency guidance & outlook

Upcoming changes

2025: Group performance

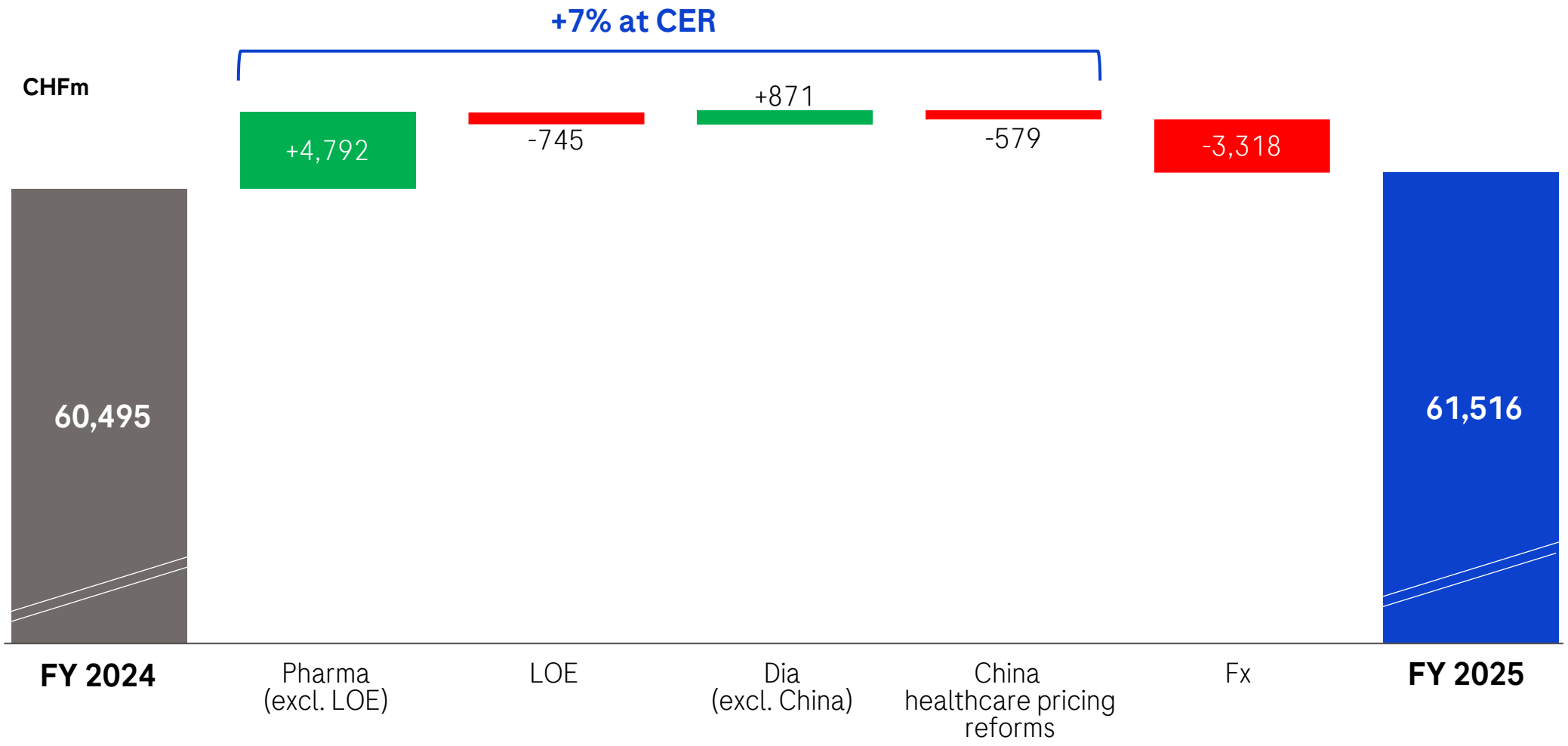
Sales increase of +7% and Core EPS increase of +11%

	2025 CHFm	2024 CHFm	Change in % CHF	Change in % CER
Sales	**61,516**	**60,495**	**+2**	**+7**
Core operating profit	**21,833**	**20,823**	**+5**	**+13**
as % of sales	*35.5*	*34.4*		
Core net income	**16,575**	**16,011**	**+4**	**+11**
as % of sales	*26.9*	*26.5*		
Core EPS (in CHF)	**19.46**	**18.80**	**+4**	**+11**
IFRS net income	**13,799**	**9,187**	**+50**	**+58**
as % of sales	*22.4*	*15.2*		
Operating free cash flow	**16,163**	**20,121**	**-20**	**-12**
as % of sales	*26.3*	*33.3*		
Free cash flow	**11,807**	**15,336**	**-23**	**-15**
as % of sales	*19.2*	*25.4*		

All numbers in CHFm, except core EPS in CHF; CER: Constant exchange rates (avg. full year 2024); EPS: Earnings per share

2025: Group sales growth at +7%

Continued strong Pharma sales driving Group growth



CER: Constant exchange rates (avg. full year 2024); LOE: Loss of exclusivity includes global losses of Avastin, Herceptin, MabThera/Rituxan, Esbriet, Lucentis and Actemra

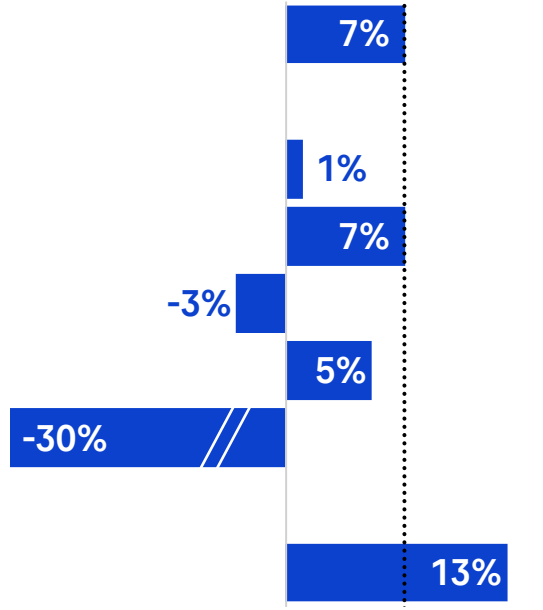
2025: Group Core operating profit

Core operating profit higher by +13%, driven by higher sales and effective cost management

	2025	
	CHFm	Abs. CERm
Sales	**61,516**	**+4,339**
Other revenue	1,840	+15
Cost of sales	-15,864	-1,136
R&D	-12,243	+332
SG&A	-13,838	-649
OOI&E	422	-188
Core operating profit	**21,833**	**+2,714**
Core OP as % of sales	*35.5%*	
At CER	*36.4%*	
	(2024: 34.5%)	

2025 vs. 2024

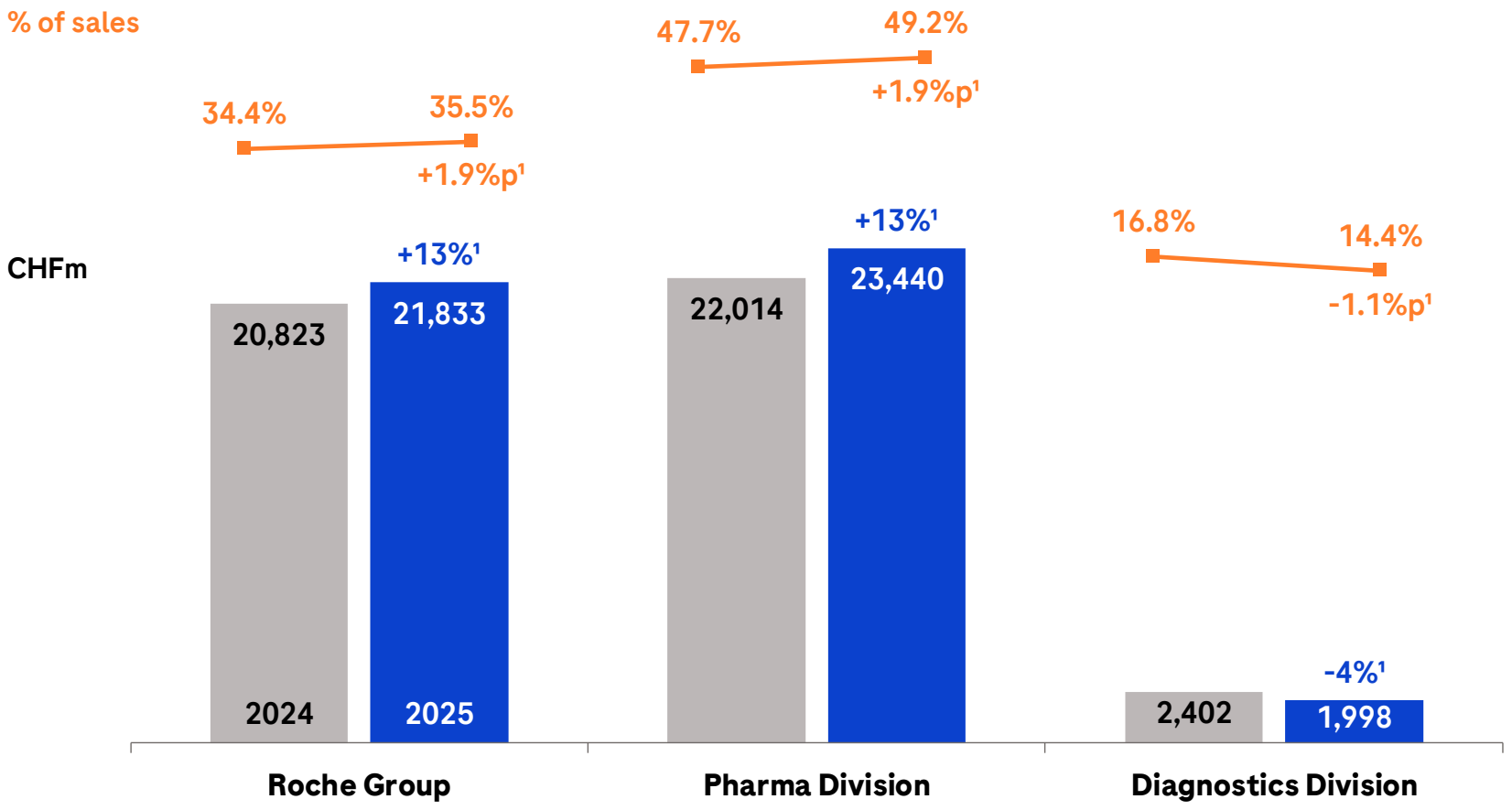
CER growth



+5% in CHF

Totals & sub-totals may include differences due to rounding; CER: Constant exchange rates (avg. full year 2024); OOI&E: Other operating income & expense; OP: Operating profit; R&D: Research & development; SG&A: Selling, general & administration

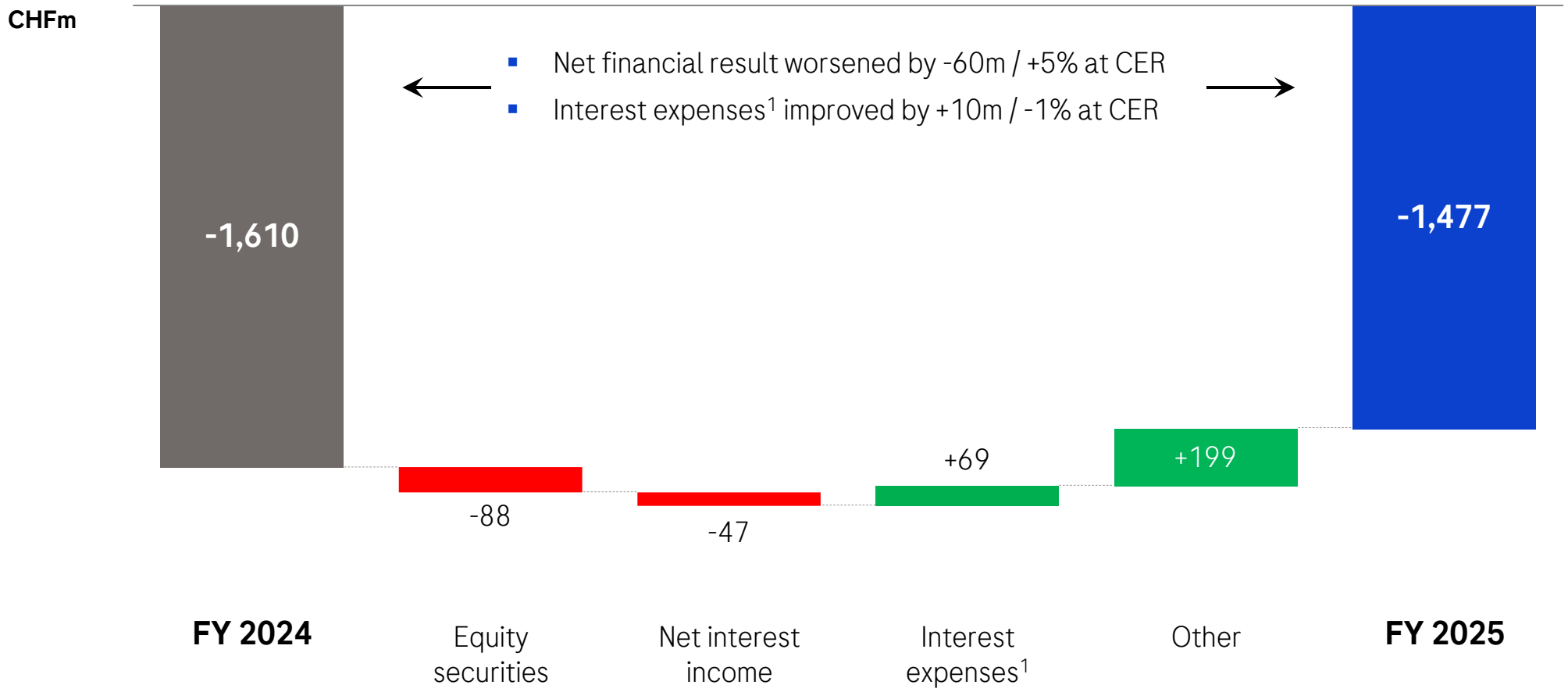
2025: Core operating profit and margin



Note: Group Core operating profit includes -3.6bn from Corporate (-3.6bn in 2024); 1. At CER: Constant exchange rates (avg. full year 2024)

2025: Core net financial result

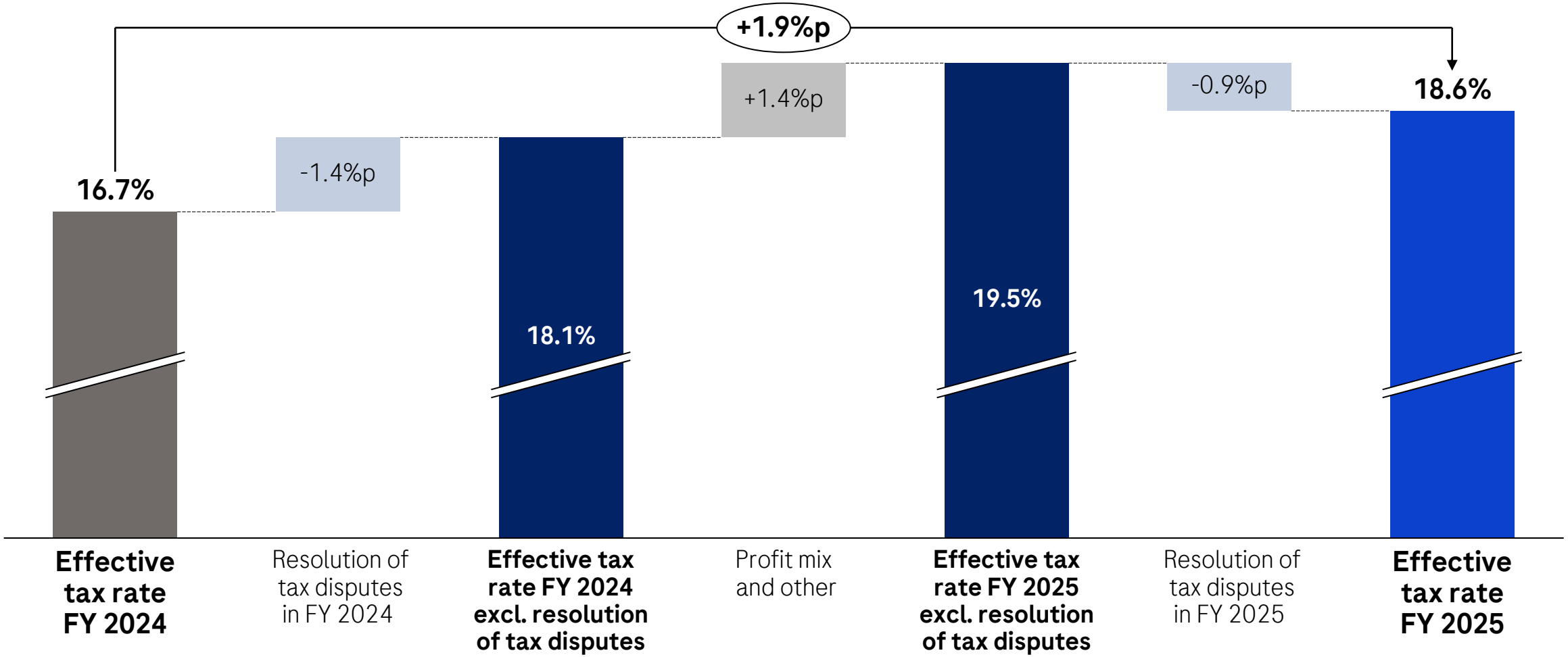
Improvement mainly driven by lower losses from foreign exchange results (Other)



1. Incl amortization of debt discount and net gains on interest rate derivatives; CER: Constant exchange rates (avg. full year 2024)

2025: Core tax rate

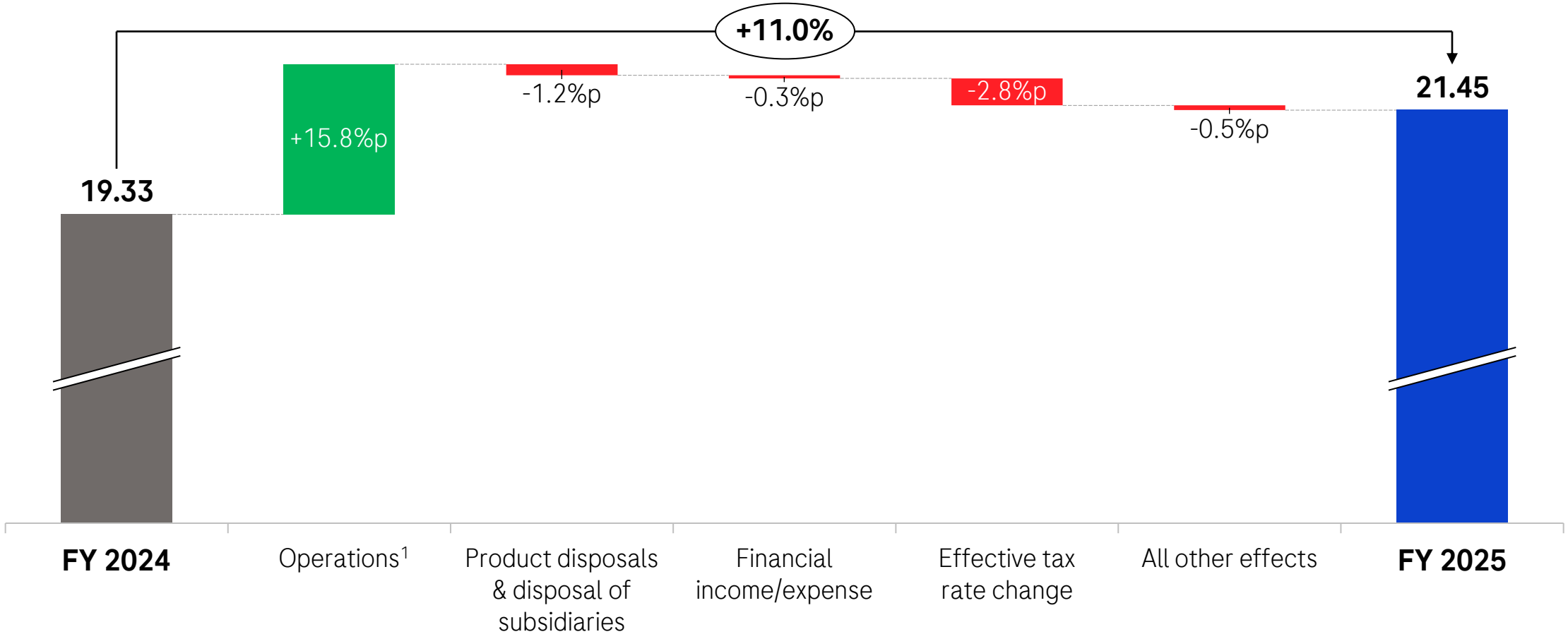
Higher tax rate mainly due to changes in profit mix and lower impact from resolution of tax disputes



All at reported exchange rates

2025: Core EPS

Increase in operations partially offset by a higher tax rate and lower gains on product disposals



All at CER: Constant exchange rates (avg. full year 2024); 1. Core operating profit excl. gains/losses on product disposals and gains from disposal of subsidiaries (InterMune sale)

2025: Non-core and IFRS income

Non-core items lower vs. PY due to lower impairment of goodwill

	2025 CHFm	2024 CHFm	Var. at CERm	Change in % CHF	Change in % CER
Core operating profit	**21,833**	**20,823**	**+2,714**	**+5**	**+13**
Global restructuring plans	-2,164	-1,891	-344		
Amortisation of intangible assets	-669	-736	+44		
Impairment of intangible assets[1]	-362	-4,631	+4,169		
M&A and alliance transactions	8	-72	+76		
Legal & environmental[2]	-170	-76	-96		
Total non-core operating items	-3,357	-7,406	+3,849		
IFRS operating profit	**18,476**	**13,417**	**+6,563**	**+38**	**+48**
Total financial result & taxes	-4,677	-4,230	-901		
IFRS net income	**13,799**	**9,187**	**+5,662**	**+50**	**+58**

1. Incl. goodwill; 2. Incl. pension plan settlements; CER: Constant exchange rates (avg. full year 2024)

Results

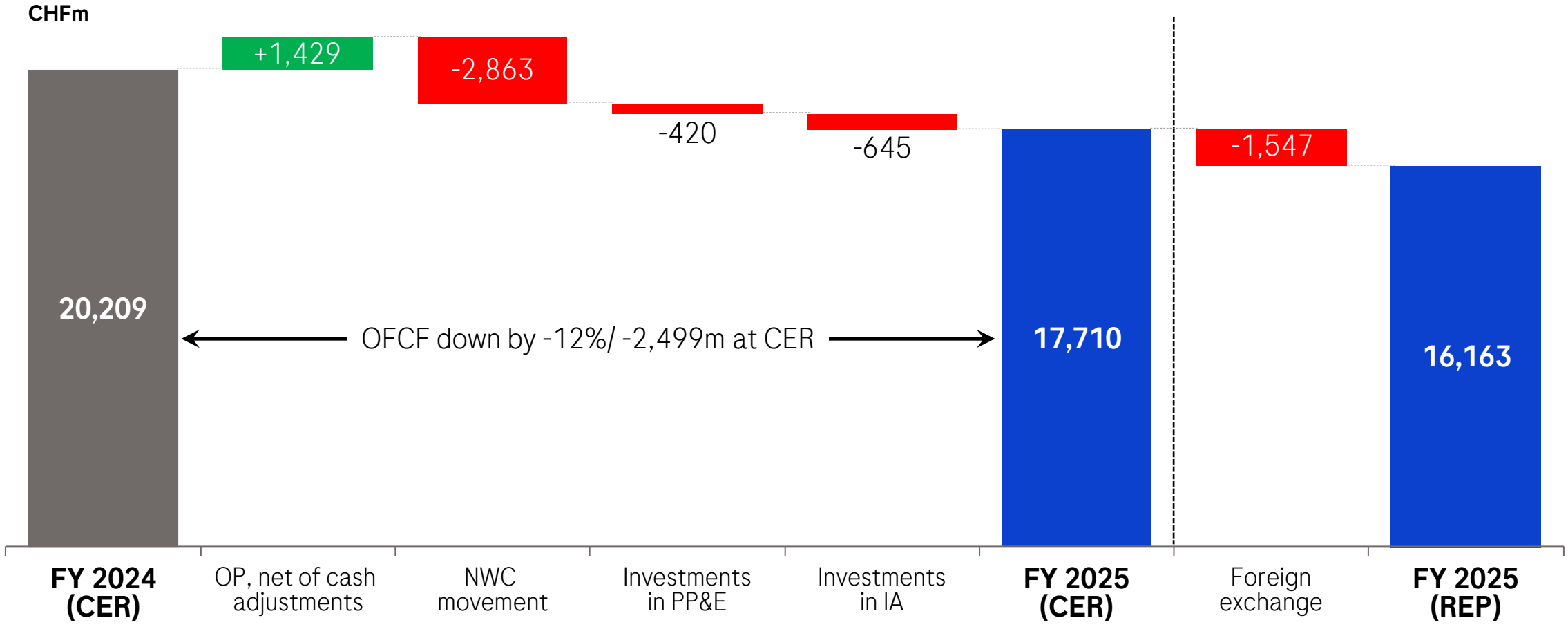
Cash & balance sheet

Currency guidance & outlook

Upcoming changes

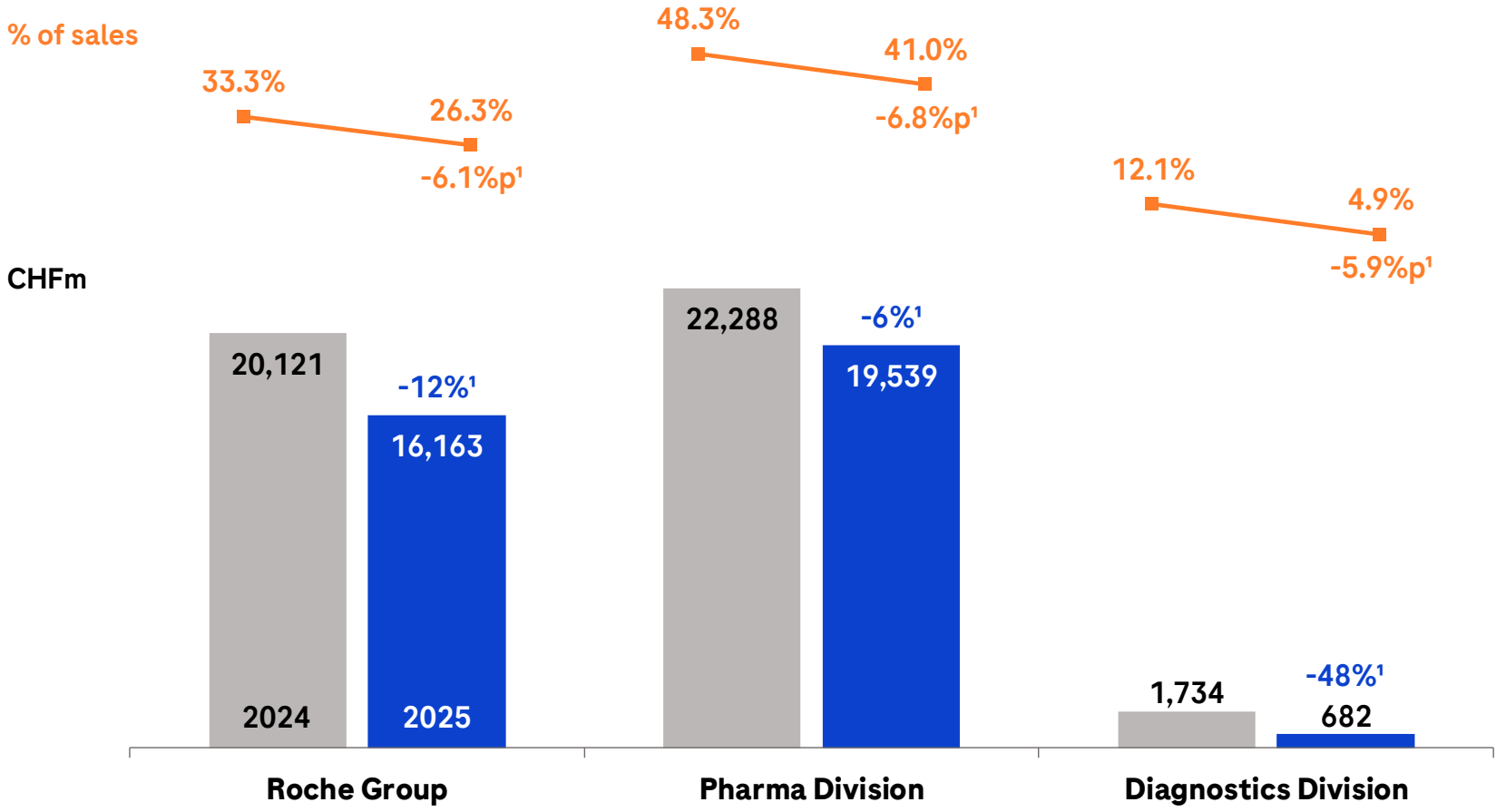
2025: Group operating free cash flow

OFCF down by -12% driven by NWC movement (higher year end receivables) and investments in IA (Zealand Pharma)



CER: Constant exchange rates (avg. full year 2024); IA: Intangible assets; NWC: Net working capital; OP: Operating profit; PP&E: Property, plant & equipment incl. lease liability paid

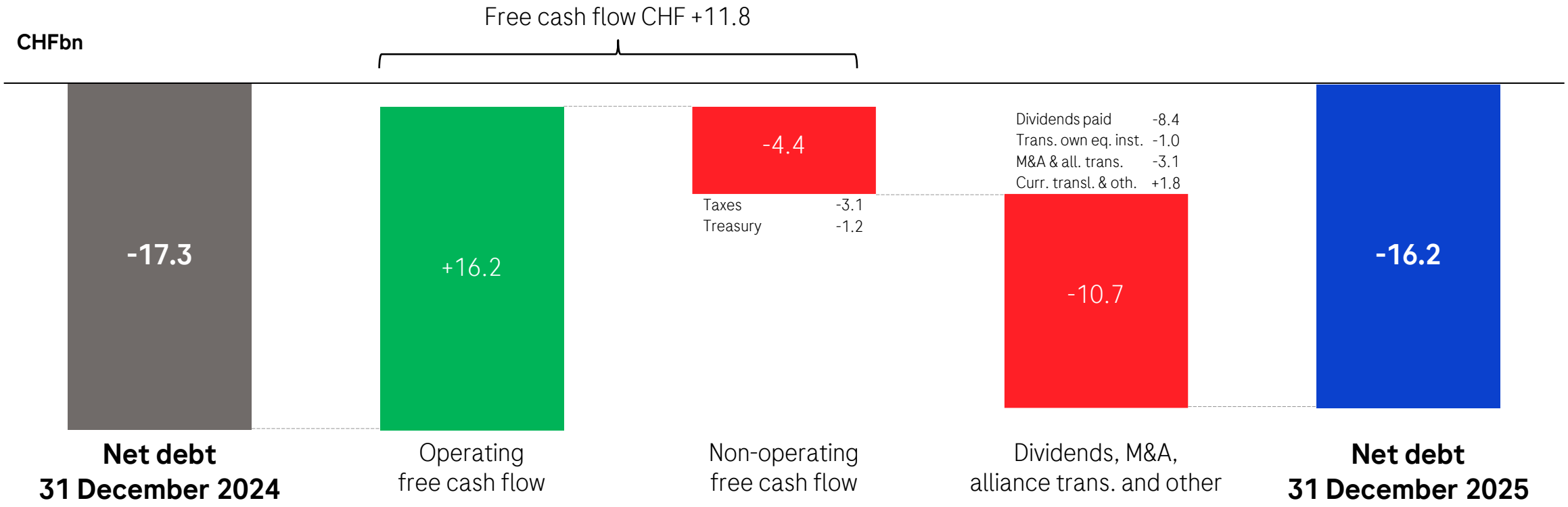
2025: Operating free cash flow and margin



Note: Group operating free cash flow includes -4.1bn from Corporate (-3.9bn in 2024); 1. At CER: Constant exchange rates

2025: Group net debt development

Net debt lower by CHF 1.2bn vs. YE 2024

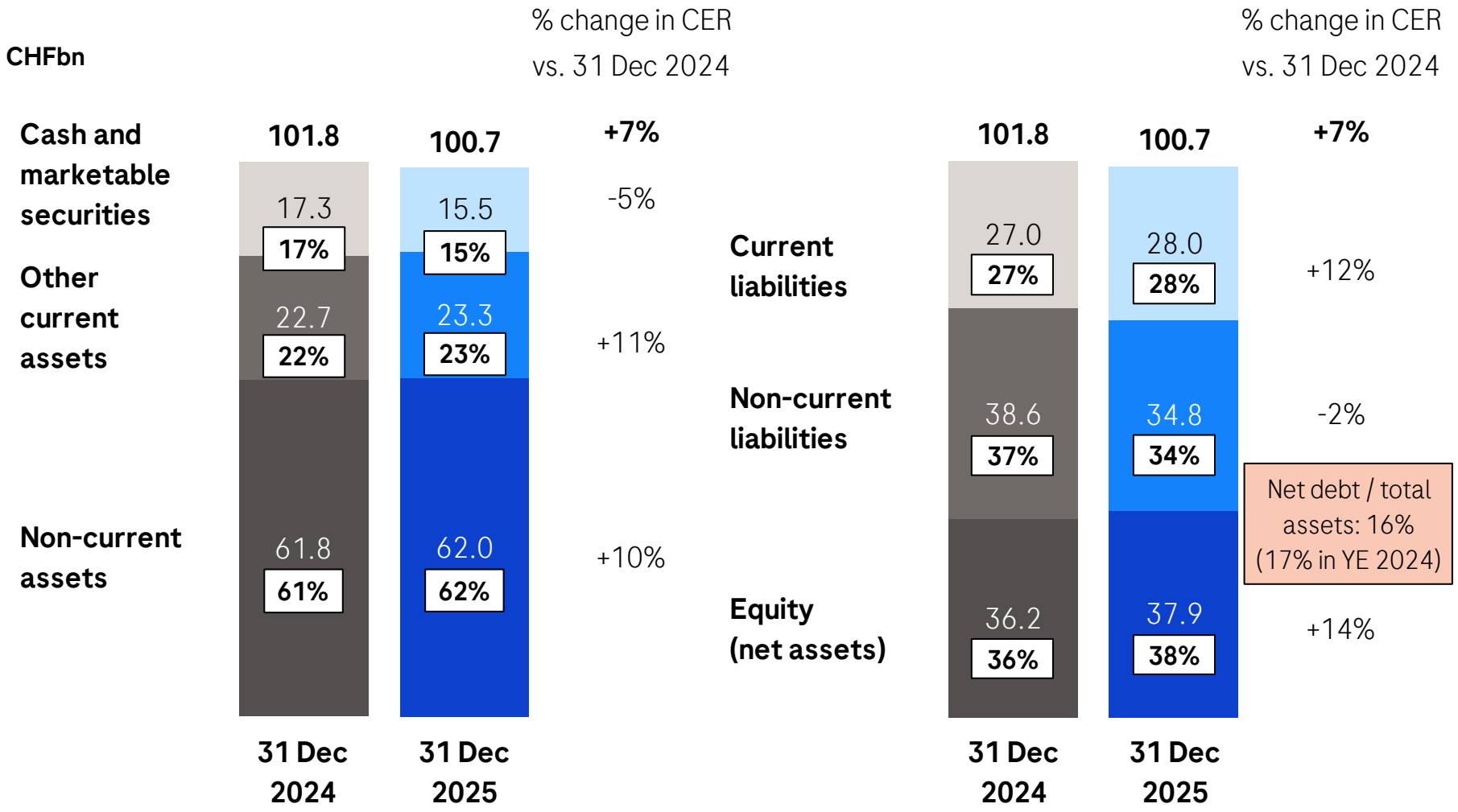


Thereof investments in innovation:		Intangible asset	Equity	M&A	Total
	2025	-2.0	0.0	-3.1	-5.1
	2024	-1.5	0.0	-3.1	-4.6

Totals & sub-totals may include differences due to rounding; M&A: Mergers & acquisitions

Balance sheet 31 December 2025

Equity ratio at 38% (31 Dec 2024: 36%)



Totals may include differences due to rounding; CER: Constant exchange rates (avg. full year 2024)

Results

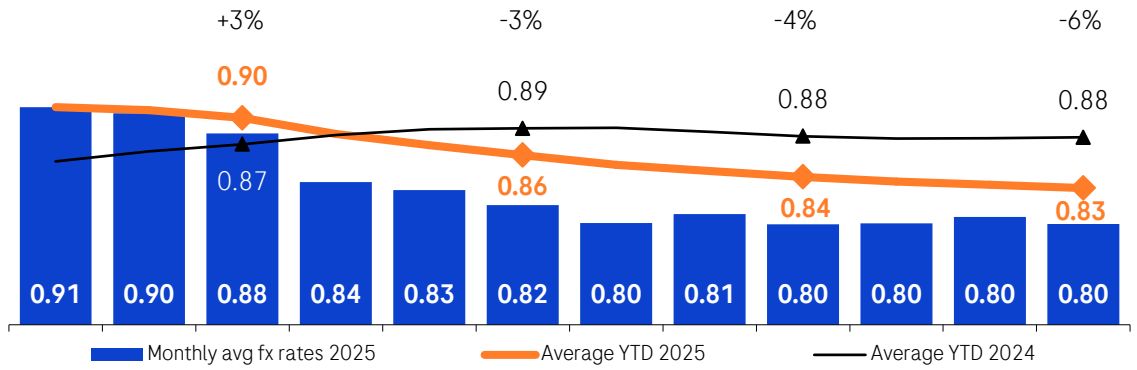
Cash & balance sheet

Currency guidance & outlook

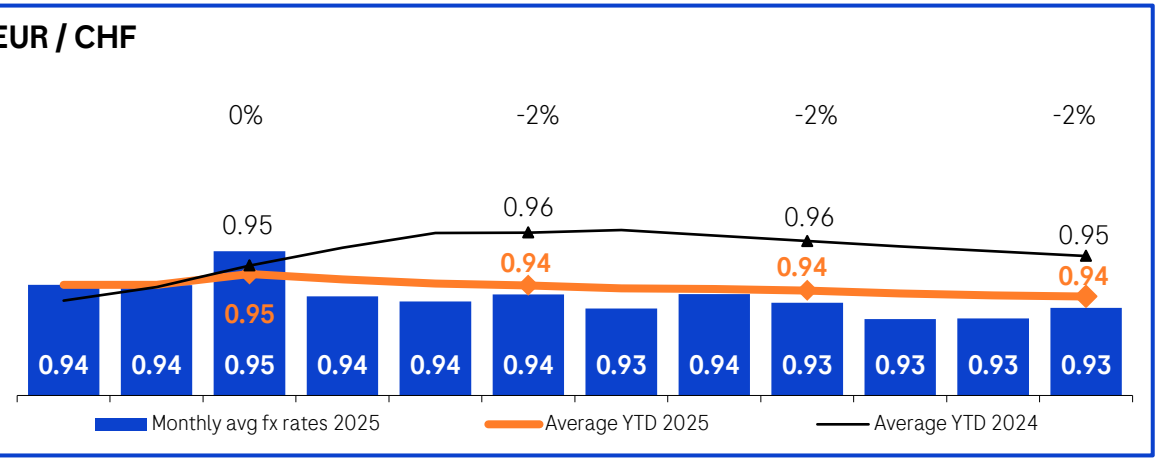
Upcoming changes

2025: Currency impact and outlook

USD / CHF



EUR / CHF



In 2025 impact[1] is (%p):

	Q1	Q2	Q3	Q4
Sales	**+1**	**-8**	**-7**	**-7**

	Q1	HY	YTD Sep	FY
Sales	**+1**	**-3**	**-5**	**-5**
Core operating profit		**-5**		**-8**
Core EPS		**-4**		**-7**

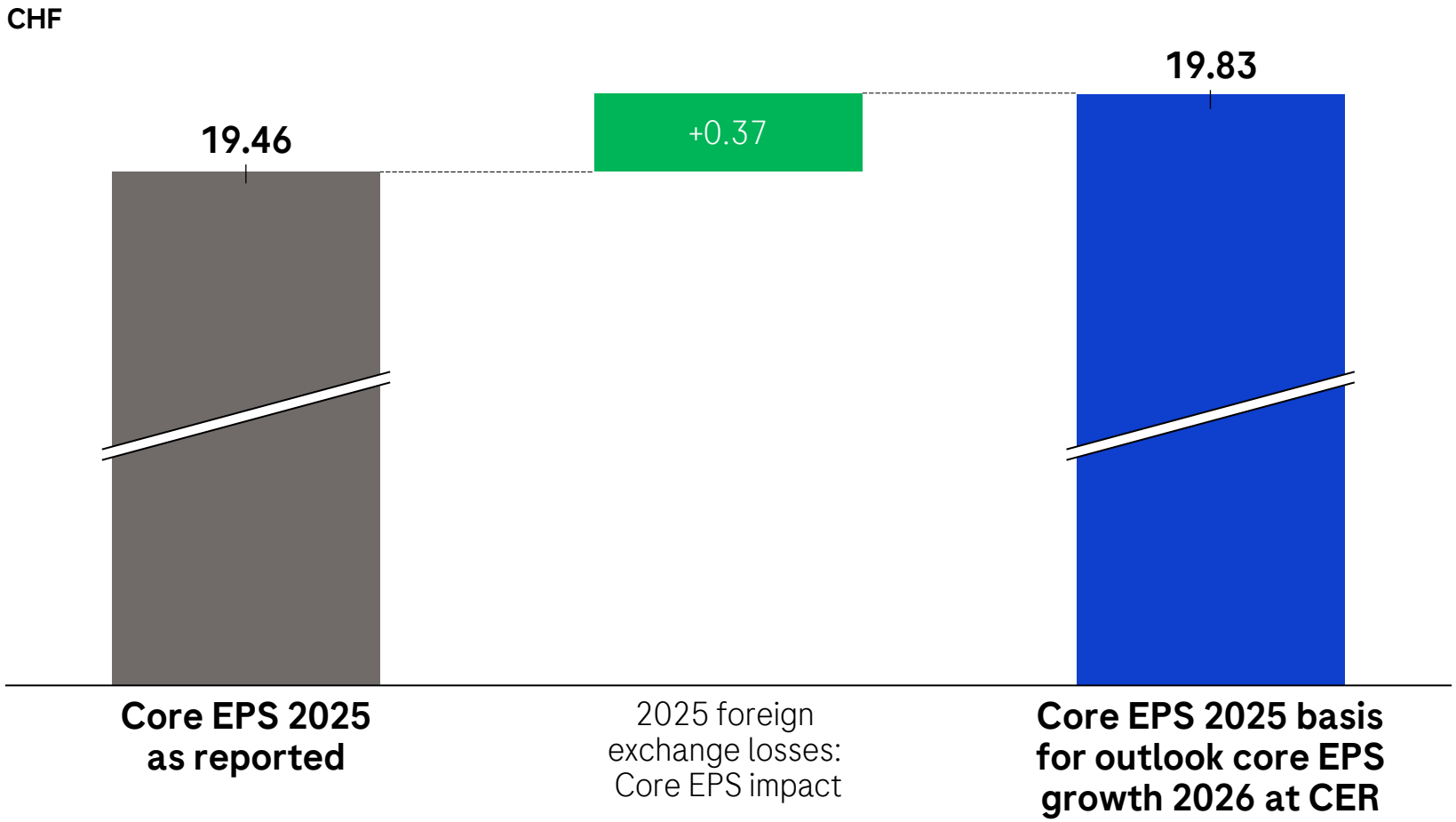
2026 currency impact expected[1] *(based on 31 Dec 2025 FX rates):*

Around -4%p on sales, -6%p on Core OP and -6%p on Core EPS

1. On Group growth rates; EPS: Earnings per share; OP: Operating profit

2025: Core EPS

Core EPS 2025 of CHF 19.83 is basis for Core EPS outlook 2026 at CER



CER: Constant exchange rates (avg. full year 2025); EPS: Earnings per share

2026 guidance

LOE impact of roughly CHF 1bn expected for 2026

Group sales growth[1]	Mid single digit sales growth
Core EPS growth[1]	High single digit Core EPS growth
Dividend outlook	Further increase dividend in Swiss francs

1. At CER: Constant exchange rates (avg. full year 2025); LOE: Loss of exclusivity includes global losses of Avastin, Herceptin, MabThera/Rituxan, Lucentis and Actemra

Results

Cash & balance sheet

Currency guidance & outlook

Upcoming changes

Prospective change in income statement presentation 2026

Further centralization of functions with global accountability: Legal costs to be reported as SG&A Corporate

Changes in Income Statement presentation

- Implementation effective 1 January 2026: Most legal costs reported as SG&A under Corporate
- No restatement of comparative 2025 information

Consequences

- Sales, Group Core operating profit, and Core EPS metrics are unaffected
- Change will reduce reported administration costs within SG&A on divisional level and will increase divisional Core OP margins at CER
  - **Pharma: ~250m less SG&A costs at CER / ~+0.5%p COP margin at CER***
  - **Dia: ~50m less SG&A costs at CER / ~+0.4%p COP margin at CER***
  - **Corporate: ~300m more SG&A costs at CER***

* Estimate based on 2025 actuals; CER: Constant exchange rates; COP: Core operating profit; COS: Cost of sales; HR: Human resources; R&D: Research & development; SG&A: Selling, general & admin



Pharmaceuticals Division

Teresa Graham
CEO Roche Pharmaceuticals

Roche Pharma: Key priorities

Sustain momentum through commercial excellence, key launches and portfolio rejuvenation



Maximize on-market portfolio

Current on-market portfolio to deliver growth until 2028

*16 blockbusters with strong momentum across TAs**



Deliver key launches

Upcoming launches to support growth momentum starting this year

giredestrant, fenebrutinib, Gazyva, vamikibart, Enspryng



Progress post Bar NMEs

Prepare entry into broad scale DAs, e.g., Alzheimer's, Obesity

Broad and diversified pipeline with transformational potential



Discipline in the business



Rigour in the science

*Based on FY 2025 sales with blockbusters= Medicines with CHF >1bn annual sales. Blockbuster count includes Venclexta, for which AbbVie books sales; DA: Disease areas; NME: New molecular entity; TA: Therapeutic area

2025: Pharmaceuticals sales

Continued strong growth across all regions

	FY 2025 CHFm	FY 2024 CHFm	Change in % CHF	Change in % CER
Pharmaceuticals Division	**47,669**	**46,171**	**3**	**9**
United States	25,355	24,774	2	8
Europe	9,164	8,832	4	5
Japan	2,882	2,874	0	5
International	10,268	9,691	6	14

CER: Constant exchange rates (avg. full year 2024)

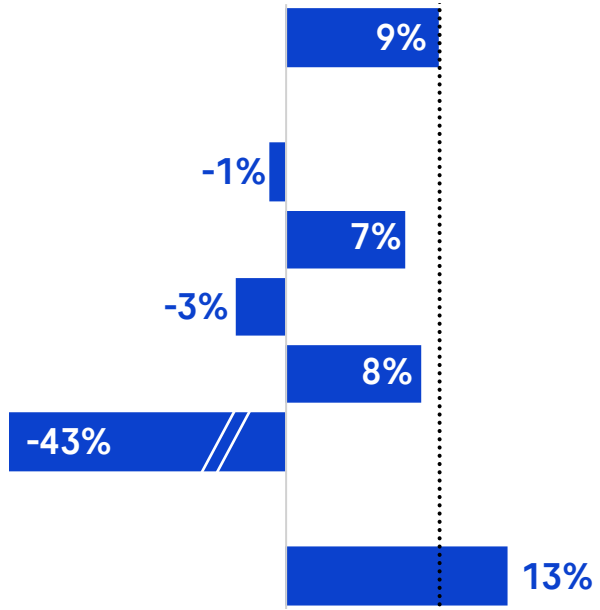
2025: Pharmaceuticals Core operating profit

Core operating profit outgrowing sales with +13%, driven by higher sales and effective cost management

	2025	
	CHFm	Abs. CERm
Sales	**47,669**	**+4,047**
Other revenue	1,782	-14
Cost of sales	-8,678	-624
R&D	-10,399	+293
SG&A	-7,241	-560
OOI&E	307	-242
Core operating profit	**23,440**	**+2,900**
Core OP as % of sales	*49.2%*	
At CER	*49.7%*	
	(2024: 47.8%)	

2025 vs. 2024

CER growth

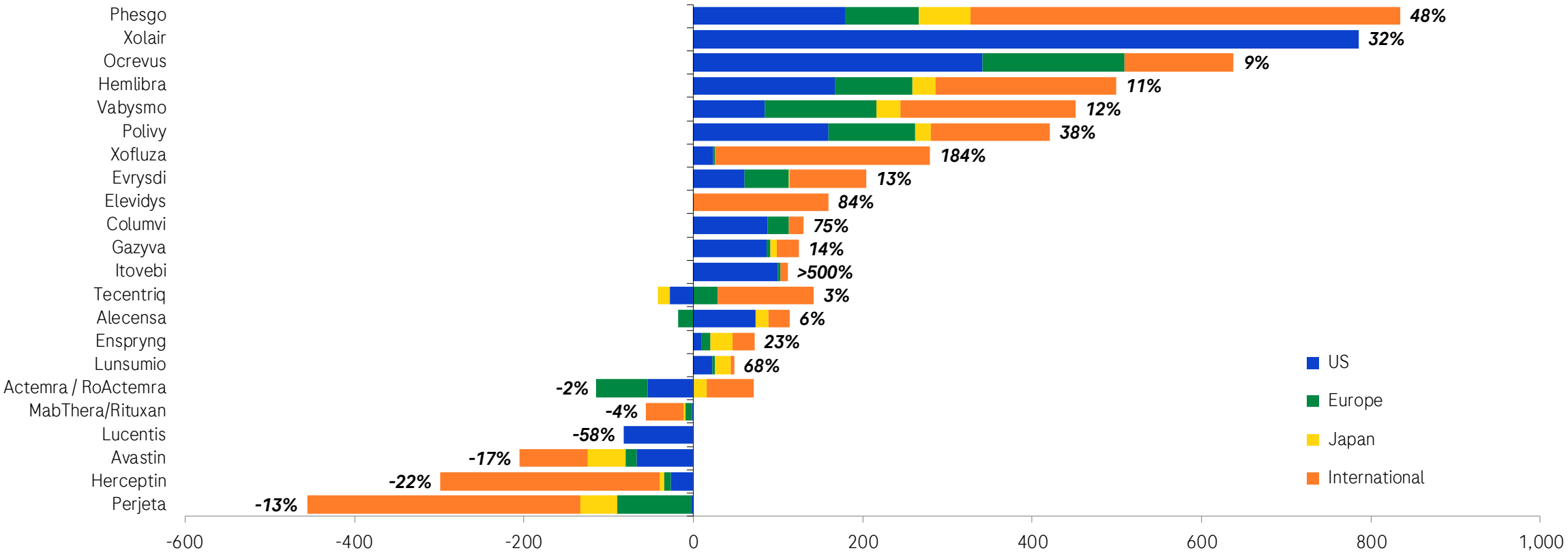


+6% in CHF

CER: Constant exchange rates (avg. full year 2024); OOI&E: Other operating income & expense; OP: Operating profit; R&D: Research & development; SG&A: Selling, general & administration

2025: Diversified set of key growth drivers

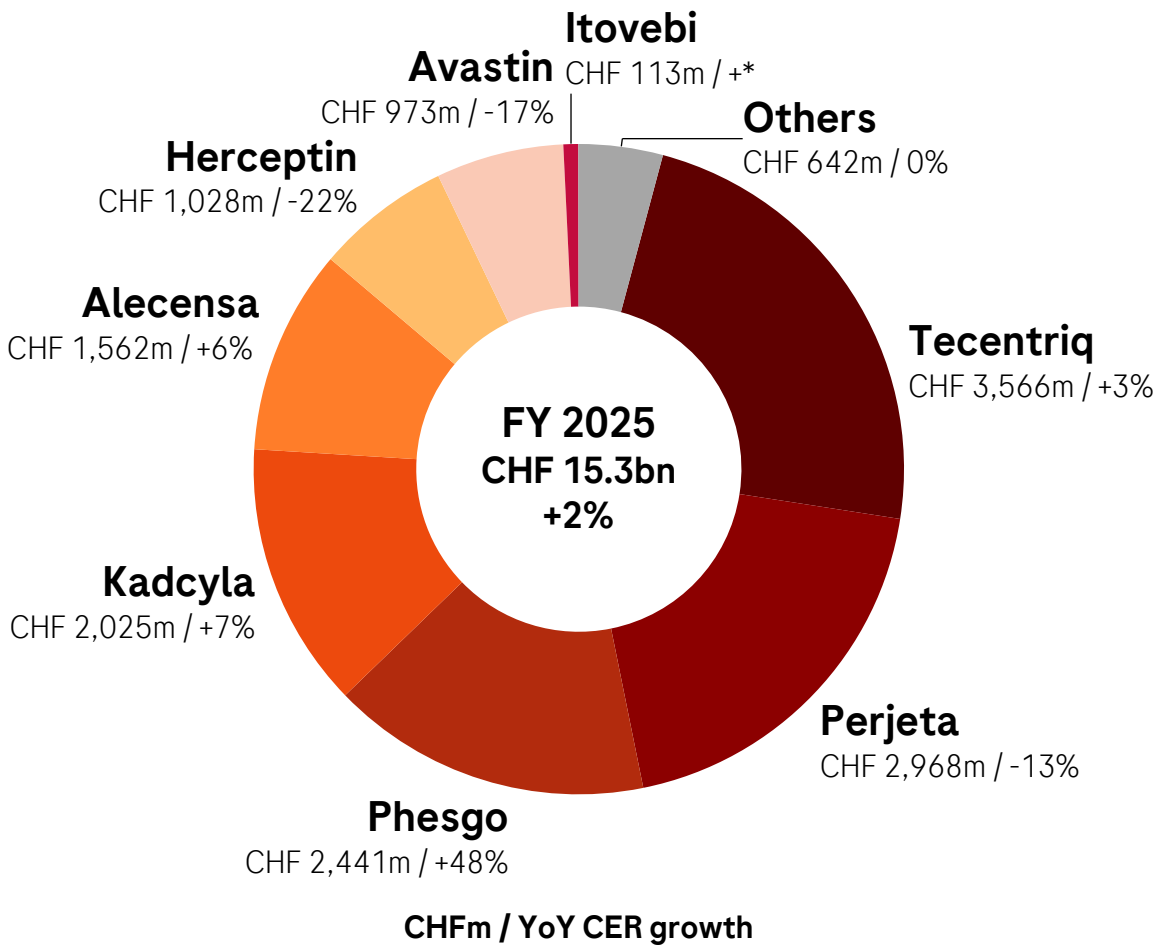
Phesgo, Xolair, Ocrevus, Hemlibra, Vabysmo, and Polivy driving growth



Absolute values and growth rates at CER: Constant exchange rates (avg. full year 2024)

Oncology growing +2% driven by the HER2+ franchise

Giredestrant: evERA US filing achieved and positive Ph III (lidERA) in adj. ER+/HER2- BC presented at SABCS



CHFm / YoY CER growth

Q4 update

- Phesgo: Global conversion rate climbing to 54%**
- Perjeta: Conversion to Phesgo ongoing
- Kadcyla: Growth driven by adjuvant BC
- Itovebi: Launch in 1L *PIK3CA*-mut HR+ BC ongoing
- Tecentriq: Return to growth due to new indications
- Alecensa: Growth driven by US & Japan
- Giredestrant: Positive Ph III (lidERA) in adj. ER+/HER2- BC presented at SABCS; US filing in post-CDKi ER+/HER2- mBC (evERA) achieved

Outlook 2026

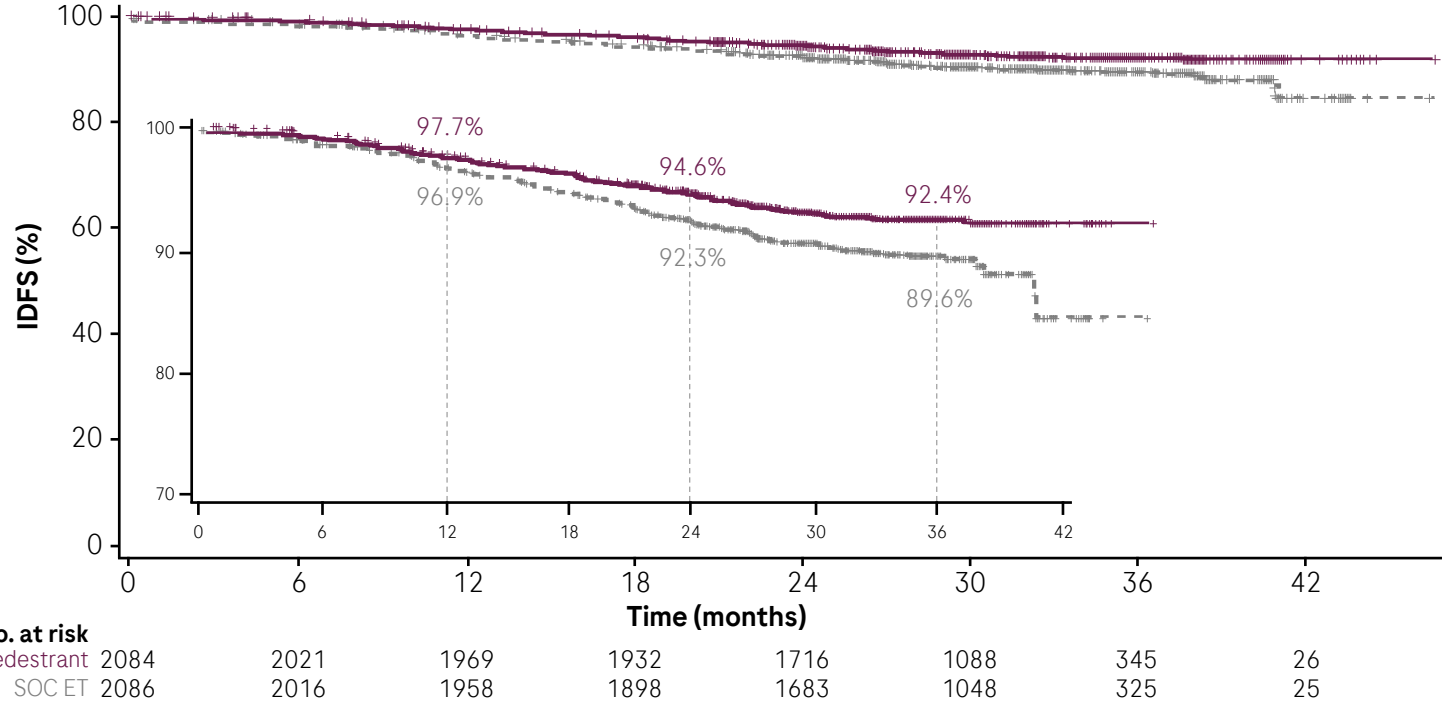
- Giredestrant in adjuvant ER+/HER2- BC: US/EU filing
- Giredestrant in post-CDKi ER+/HER2- mBC: US approval/EU filing
- Ph III (persevERA) giredestrant in 1L ER+/HER2- mBC
- Ph III (INAVO121) Itovebi in post CDKi *PIK3CA*-mut HR+/HER2- mBC
- Ph III (INAVO122) Itovebi + Phesgo in 1L *PIK3CA*-mut HER2+ mBC
- Ph III (KRASCENDO 1) divarasib in 2L KRASG12C+ NSCLC

* growth >500%; ** Perjeta/Phesgo conversion rate calculated using volumes, currently taking 80 launch countries into account (78 countries at Q3 2025); CDK4/6i: Cyclin dependent kinase 4/6 inhibitor; CER: Constant exchange rates (avg. full year 2024); ER: Estrogen receptor; HER2: Human epidermal growth factor 2; HR: Hormone receptor; (m)BC: (Metastatic) breast cancer; NSCLC: Non-small cell lung cancer; PIK3CA-mut: Phosphatidylinositol 3-kinase, catalytic, alpha polypeptide mutated

Giredestrant: Positive Ph III (lidERA) results in adjuvant ER+/HER2- BC

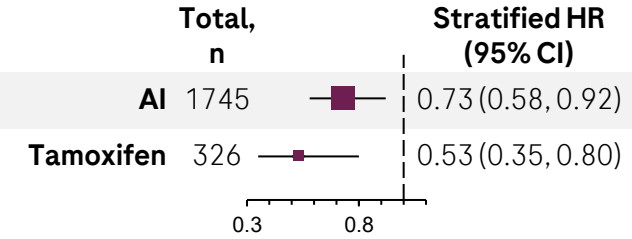
First and only oral SERD to show superior iDFS in the adjuvant setting

Ph III (lidERA) iDFS results in adjuvant ER+/HER2- BC[1]



	Giredestrant n = 2084	SOC ET n = 2086
Events, n (%)	140 (6.7)	196 (9.4)
Stratified HR (95% CI)	**0.70** (0.57, 0.87); **p = 0.0014***	

Exploratory analysis: IDFS by SOC ET



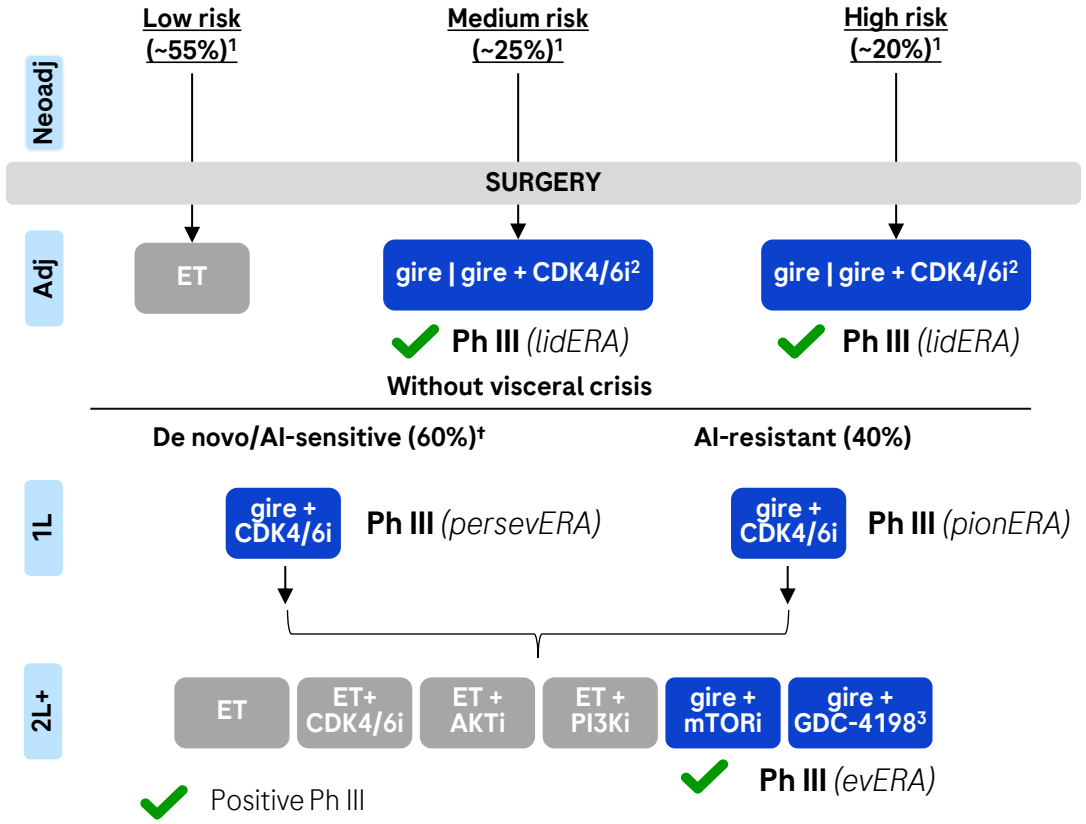
- Giredestrant demonstrated a statistically significant and clinically meaningful improvement in iDFS (HR 0.70) in adj. ER+/HER2- BC
- OS data were immature but showed a clearly favorable trend for giredestrant (HR 0.79)
- Safety profile was favorable and consistent with the known profile; lower discontinuation rate with giredestrant compared to SOC ET

1. Bardia et al., SABCS 2025; BC: Breast cancer; CI: Confidence interval; ER: Estrogen receptor; ET: Endocrine therapy; HER2: Human epidermal growth factor 2; HR: Hazard ratio; iDFS: invasive disease-free survival; OS: Overall survival; SERD: Selective estrogen receptor degrader; SOC: Standard of care

Giredestrant: Next-gen SERD with potential to replace SOC ET in ER+ BC

Complemented by a portfolio of potentially BIC molecules allowing combinations

Potential to redefine treatment paradigm in ER+ BC



Differentiated molecules targeting key signaling nodes

ER

Giredestrant: Next generation oral SERD with the potential to become the new ET backbone

PI3K AKT mTOR

Itovebi: PI3K inhibitor with greater safety margins allowing for combination with ET and CDK4/6i at standard doses with proven substantial benefit

CDK

RG6794 (GDC-4198): Highly potent CDK4 inhibitor with substantial activity against CDK2 to address a key mechanism of resistance to CDK4/6i

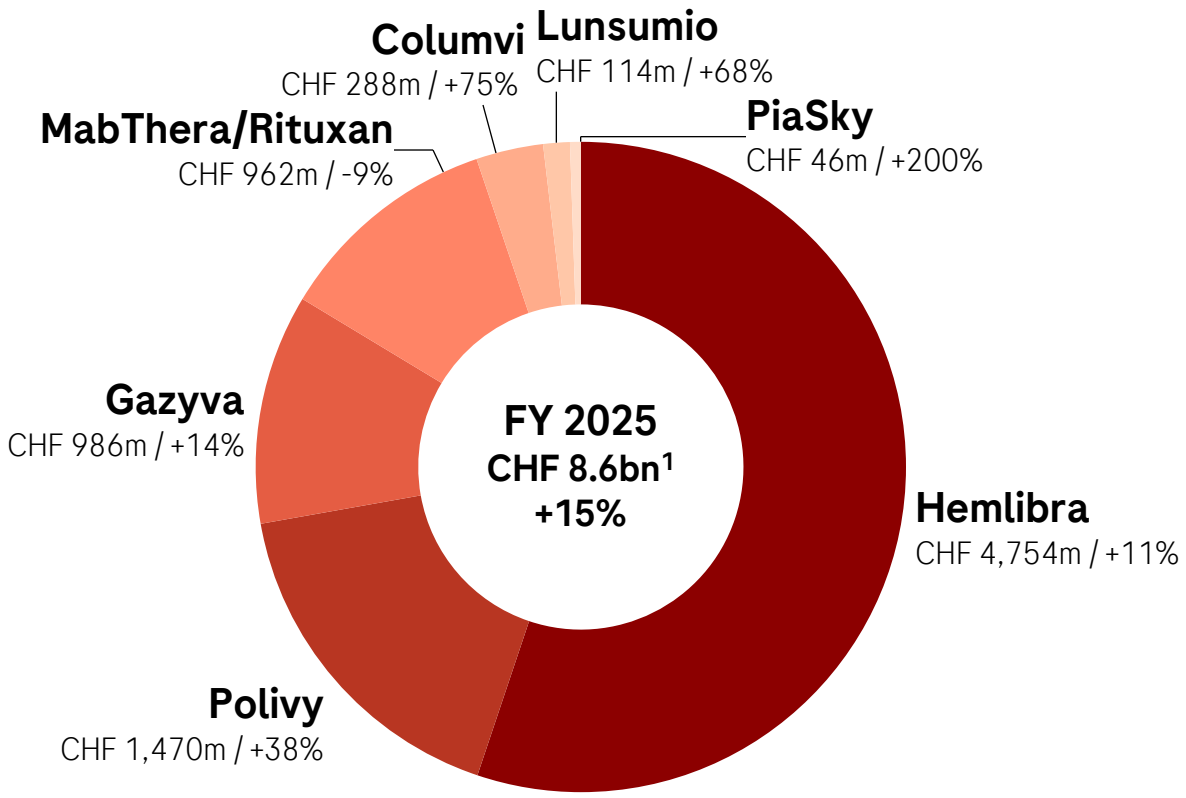
HER2

ZN-1041: Highly selective, brain-penetrant HER2 TKI, potentially combinable with established anti-HER2 backbones

1. Risk definitions vary according to guidelines and tools used: stage at diagnosis based on internal estimates using SEER data; 2. giredestrant + CDK4/6i in adjuvant ER+ BC being evaluated as single arm substudy as part of Ph III lidERA; 3. Ph II trial: NCT07100106; †AI sensitive defined as patients who relapse >1yr after completion of adjuvant therapy; AI: Aromatase inhibitor; BC: Breast cancer; BIC: Best in Class; CDK: Cyclin-dependent kinase; ER: Estrogen receptor; ET: Endocrine therapy; HER2: Human Epidermal growth factor receptor 2; PI3K: Phosphoinositide 3-kinase; Neoadj: Neoadjuvant; SERD: Selective estrogen receptor degrader; SOC: Standard of care; TKI: Tyrosine kinase inhibitor

Polivy US patient share in 1L DLBCL (IPI 0-5) reaching 36%

Lunsumio SC in 3L+ FL achieved US and EU approval



CHFm / YoY CER growth

Q4 update

- Hemlibra: Continued global growth; increasing adoption in non-inhibitor patients
- Polivy: Strong 1L DLBCL uptake with >80k pts treated globally
- Gazyva: Growth driven by combinations in 1L CLL
- Columvi: Driven by 2L+ DLBCL launches and 3L+ DLBCL
- Lunsumio: Strong 3L+ FL launch; US & EU approval for SC achieved
- PiaSky: Positive Ph III (COMMUTE) in aHUS

Outlook 2026

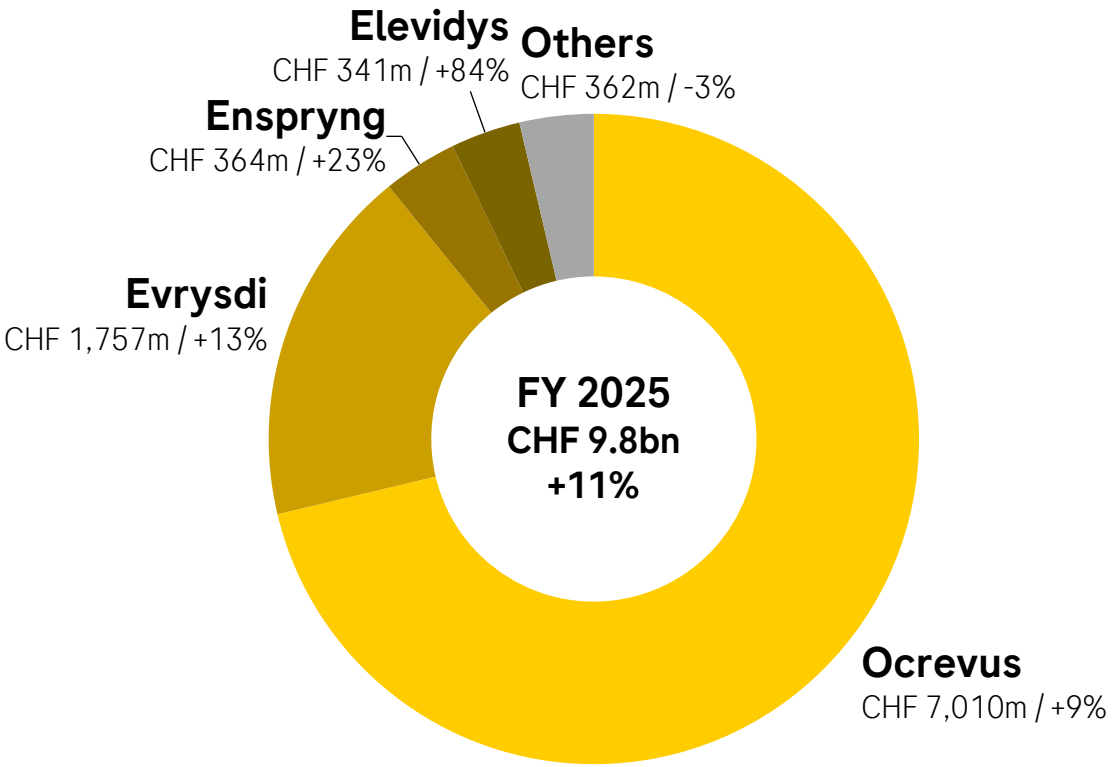
- PiaSky in aHUS: US/EU filing
- Lunsumio + Polivy in 2L+ DLBCL: US approval
- Ph III (CELESTIMO) Lunsumio + lenalidomide in 2L+ FL

1. Venclexta sales booked by AbbVie and therefore not included; aHUS: Atypical hemolytic uremic syndrome; CER: Constant exchange rates (avg. full year 2024); CLL: Chronic lymphocytic leukemia; DLBCL: Diffuse large B cell lymphoma; FL: Follicular lymphoma; SC: Subcutaneous

Ocrevus: >50% of global growth in Q4 driven by SC

First positive Ph III results for fenebrutinib in RMS (FENhance 2) and PPMS (FENtrepid)

IR Neurology/ACTRIMS Event 9 February



CHFm / YoY CER growth

Q4 update

- Ocrevus: >17,500 patients on Ocrevus SC globally
  - US: >60% of SC volume coming from community practices
  - US: ~50% of SC starts are new to brand
- Evrysdi: >21,000 patients treated globally
- Elevidys: Positive benefit-risk in ambulatory patients with >1,050 treated*; 3-year EMBARK data show durable efficacy
- Fenebrutinib in MS: Positive Ph III in RMS (FENhance 2) and PPMS (FENtrepid); PPMS results to be presented at ACTRIMS
- Prasinezumab in PD: Ph III (PARAISO) initiated
- Enspryng in MOG-AD: Positive Ph III (METEOROID); data to be presented at upcoming medical conference

Outlook 2026

- Enspryng in MOG-AD: US/EU filing
- Ph III (FENhance 1) fenebrutinib in RMS
- Ph II (MANATEE) Evrysdi + emugrobart in SMA to be presented
- Ph II (MANOEUVRE) emugrobart in FSHD to be presented

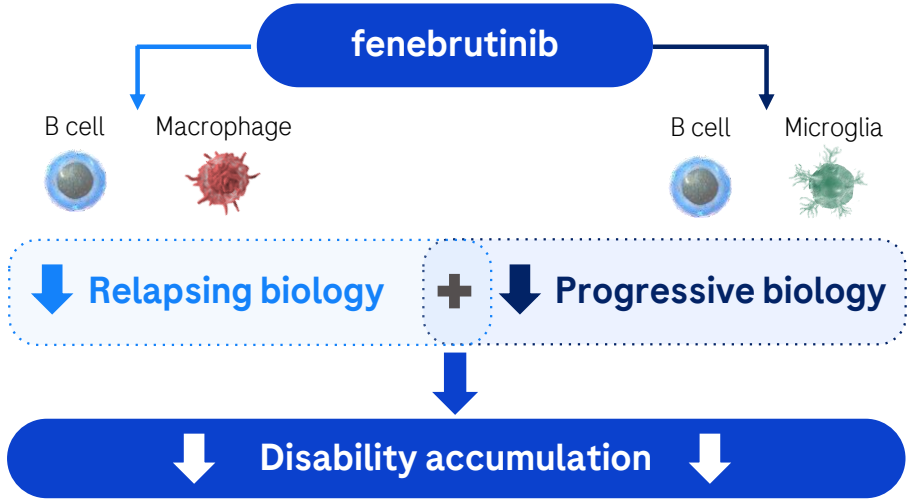
*>1,050 ambulatory patients treated across clinical and commercial setting; AD: Alzheimer's disease; CER: Constant exchange rates (avg. full year 2024); FSHD: Facioscapulohumeral muscular dystrophy; MOG-AD: Myelin oligodendrocyte glycoprotein antibody-associated disease; PD: Parkinson's disease; (R/PP)MS: (Relapsing/Primary progressive) multiple sclerosis; SC: Subcutaneous; SMA: Spinal muscular atrophy

Fenebrutinib: Potential for FIC and BIC in both RMS and PPMS

First and only BTKi with positive results in Ph III trials in RMS (FENhance 2) and PPMS (FENtrepid)

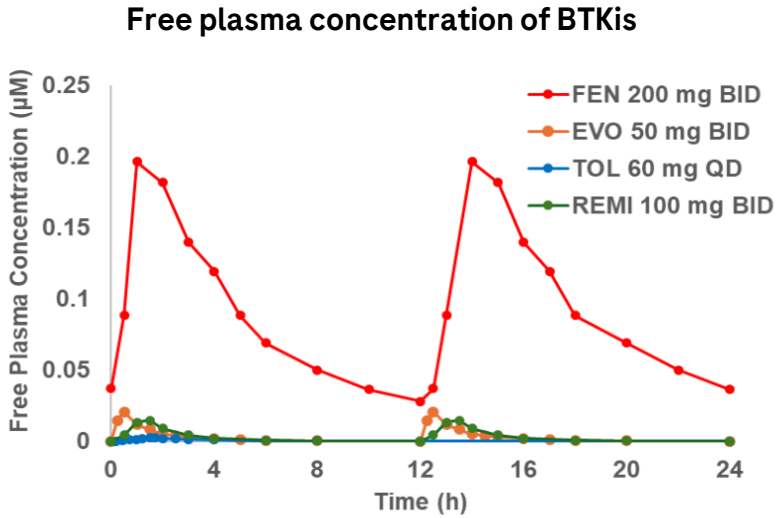
IR Neurology/ACTRIMS Event 9 February

Designed to control relapsing and progressive biology



- BTKi dual MoA has the potential to impact both relapsing and progressive disease biology in MS
- Potential first and only high-efficacy oral treatment for both RMS and PPMS
- Opportunity to increase high-efficacy treatment (HET) rates and expand Roche MS franchise footprint

Optimized PK profile[1]

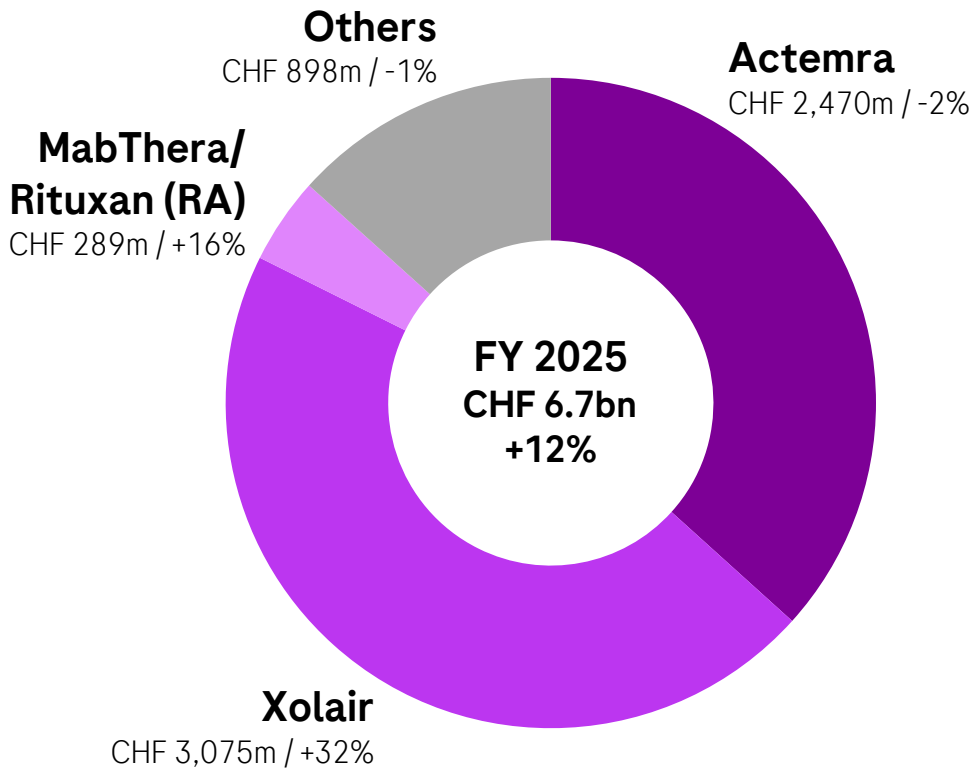


- Fenebrutinib is the only non-covalent BTKi in MS Ph III development with an optimized PK profile that allows to reach its targets[2-5]
- Fenebrutinib with high bioavailability, long half-life and large free fraction in plasma
- Free plasma AUC (drug available to cross the blood-brain barrier) significantly higher vs. other BTKi

1. Total plasma concentration-time PK profiles and PK parameters obtained from healthy volunteer populations for all BTKis. Profiles obtained by digitizing graphs. Free plasma concentrations calculated by multiplying total plasma concentrations by fraction unbound in plasma. Sources: FEN: Chen, et al. 2020; EVO: Becker, et al. 2020; TOL: Cabanis, et al. 2024; REMI: Kaul, et al. 2021; 2. Johnson AR, et al. Presented at MSVirtual 2020 (Presentation number P0338); 3. Johnson A, et al. Presented at ACTRIMS 2025 (Poster P146); 4. Crawford J, et al. J Med Chem 2018;61:2227–452; 5. Bar-Or A, et al. Presented at CMSC 2025 (Oral presentation PLA-A5); AUC: Area under curve; BID: Orally bi-daily; BIC: Best in class; FIC: First in class; (R/PP)MS: (Relapsing/Primary progressive) Multiple sclerosis; PK: Pharmacokinetics; QD: Orally once daily

Xolair strong growth momentum driven by food allergy

Gazyva: Positive Ph III results in SLE (ALLEGORY) and INS (INShore); EU approval for LN achieved



CHFm / YoY CER growth

Q4 update

- Xolair: Strong food allergy launch with >100k patients treated
  - Biosimilar launch expected H2 2026
- Actemra: Growing biosimilar impact in US
- Gazyva:
  - EU approval in LN achieved
  - Positive Ph III in SLE (ALLEGORY); submitted to SLEuro
  - Positive Ph III in INS (INShore); submitted to WCN; FDA BTD granted

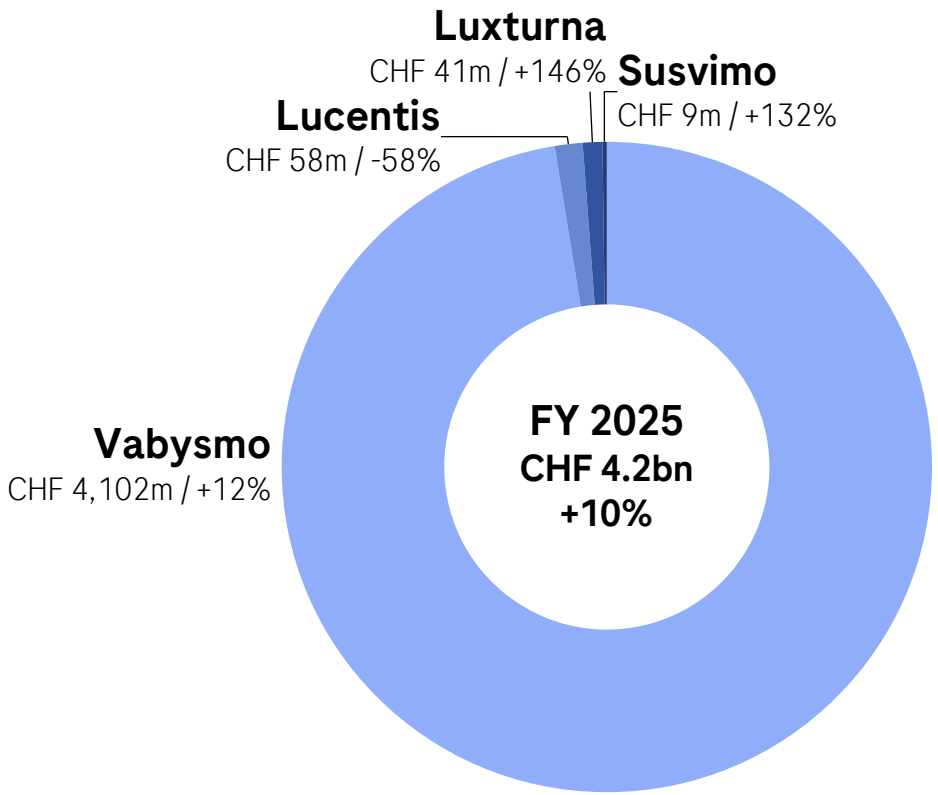
Outlook 2026

- Gazyva in SLE: US/EU filing
- Gazyva in INS: US/EU filing
- Ph III (MAJESTY) Gazyva in MN
- Ph III (IMAGINATION) sefaxersen in IgAN

CER: Constant exchange rates (avg. full year 2024); IgAN: IgA nephropathy; INS: Childhood onset idiopathic nephrotic syndrome (also known as pediatric nephrotic syndrome (PNS); LN: Lupus nephritis; MN: Membranous nephropathy; SLE: Systemic lupus erythematosus

Vabysmo continued strong global growth

Vamikibart in UME and Enspryng in TED to be filed in 2026



CHFm / YoY CER growth

Q4 update

- Vabysmo: Continued market share gains across early launch countries and ongoing global expansion
  - US branded market contracted in 2025, recovery expected in 2026
  - >60% of US patient starts are naïve

Outlook 2026

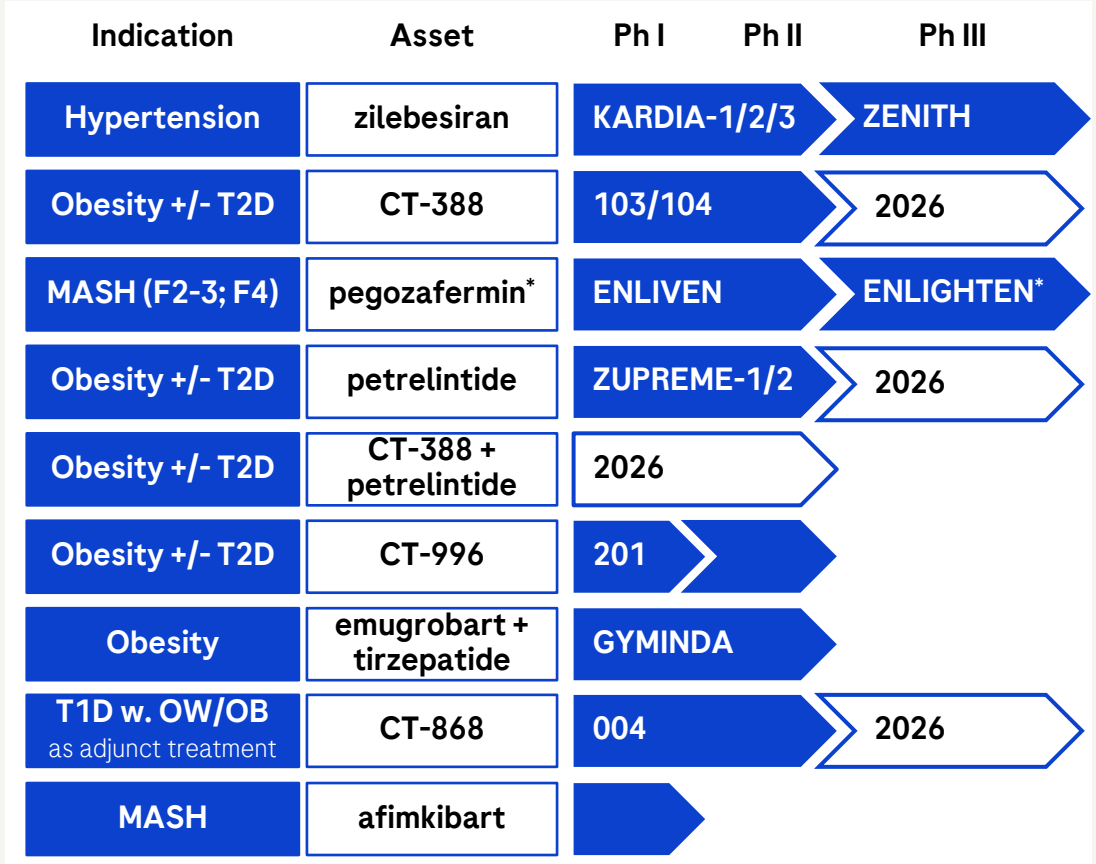
- Vamikibart in UME: US/EU filing
- Enspryng in TED: US filing
- Susvimo in nAMD: EU approval
- Susvimo in DME: EU filing
- Ph III (POYANG) Vabysmo in CNV

CER: Constant exchange rates (avg. full year 2024); CNV: Choroidal neovascularization; DME: Diabetic macular edema; nAMD: Neovascular age-related macular degeneration; TED: Thyroid eye disease; UME: Uveitic macular edema

CVRM pipeline: Ph III enabling data for five NMEs expected in 2026

Positive Ph II (103) for CT-388 in obesity w/o T2D: Placebo-adjusted WL of 22.5% at 48w (efficacy estimand) achieved

Clinical development



Q4 update

- CT-388 in obesity without T2D: Positive Ph II (103) top-line results communicated; data to be presented at upcoming conference
- CT-868 in T1D w. OW/OB as adjunct Tx to insulin: Positive Ph II (004) results; data to be presented at upcoming conference

Outlook 2026

- Ph II (104) CT-388 in obesity with T2D
- Ph II CT-996 in obesity
- Ph II (ZUPREME-1/2) petrelintide in obesity
- Ph II (GYMINDA) emugrobart + tirzepatide in obesity

- CT-388 in obesity: Ph III initiation
- CT-868 in T1D w. OW/OB ad adjunct Tx: Ph III initiation
- Petrelintide in obesity: Ph III initiation planned
- CT-996 in obesity: Ph III initiation planned
- CT-388 + petrelintide in obesity: Ph II initiation

Petrelintide in partnership with Zealand Pharma; * Ph III program consisting of ENLIGHTEN fibrosis in MASH (F2-F3) and ENLIGHTEN cirrhosis in MASH (F4); CVRM: Cardiovascular, renal and metabolism; MASH: Metabolic dysfunction-associated steatohepatitis; NME: New molecular entity; OB: Obesity; OW: Overweight; T1D/T2D: Type-1/2 diabetes; Tx: Treatment; WL: Weight loss; w: weeks

2026: Key newsflow outlook*

Giredestrant + everolimus (post CDKi ER+/HER2- mBC) filed in US

	Compound	Indication	Milestone	
Regulatory	**giredestrant**	adjuvant ER+/HER2- early BC	US/EU filing	
	giredestrant + everolimus	post CDKi ER+/HER2- mBC	EU filing/US approval	
	Gazyva	SLE	US/EU filing	
	Gazyva	INS	US/EU filing	
	Enspryng	MOG-AD	US/EU filing	
	Enspryng	TED	US filing	
	vamikibart	UME	US/EU filing	
	Susvimo	nAMD	EU approval	
	Susvimo	DME	EU filing	
	PiaSky	aHUS	US/EU filing	
	Lunsumio + Polivy	2L+ DLBCL	US approval	
Clinical results	**divarasib**	2L KRASG12C+ NSCLC	Ph III KRASCENDO 1	
	giredestrant + palbociclib	1L ER+/HER2- mBC	Ph III persevERA	
	Itovebi + fulvestrant	post CDKi *PIK3CA*-mut HR+/HER2- mBC	Ph III INAVO121	
	Itovebi + Phesgo	1L *PIK3CA*-mut HER2+ mBC	Ph III INAVO122	
	Lunsumio + lenalidomide	2L+ FL	Ph III CELESTIMO	
	Enspryng	MOG-AD	Ph III METEOROID	✓
	fenebrutinib	RMS	Ph III FENhance 1/2	__ / ✓
	Gazyva	MN	Ph III MAJESTY	
	sefaxersen	IgAN	Ph III IMAGINATION	
	Vabysmo	CNV	Ph III POYANG	
	Evrysdi + emugrobart	SMA	Ph II MANATEE	*(data inhouse)*
	emugrobart	FSHD	Ph II MANOEUVRE	*(data inhouse)*
	emugrobart + tirzepatide	Obesity	Ph II GYMINDA	
	CT-388 (QW SC)	Obesity	Ph II	✓
	CT-868 (QD SC)	T1D with Obesity	Ph II	✓
	CT-996 (QD oral)	Obesity	Ph II	
	petrelintide	Obesity	Ph II ZUPREME-1/2	

*Outcome studies are event-driven: timelines may change



Diagnostics Division

Matt Sause

CEO Roche Diagnostics

2025: Diagnostics sales

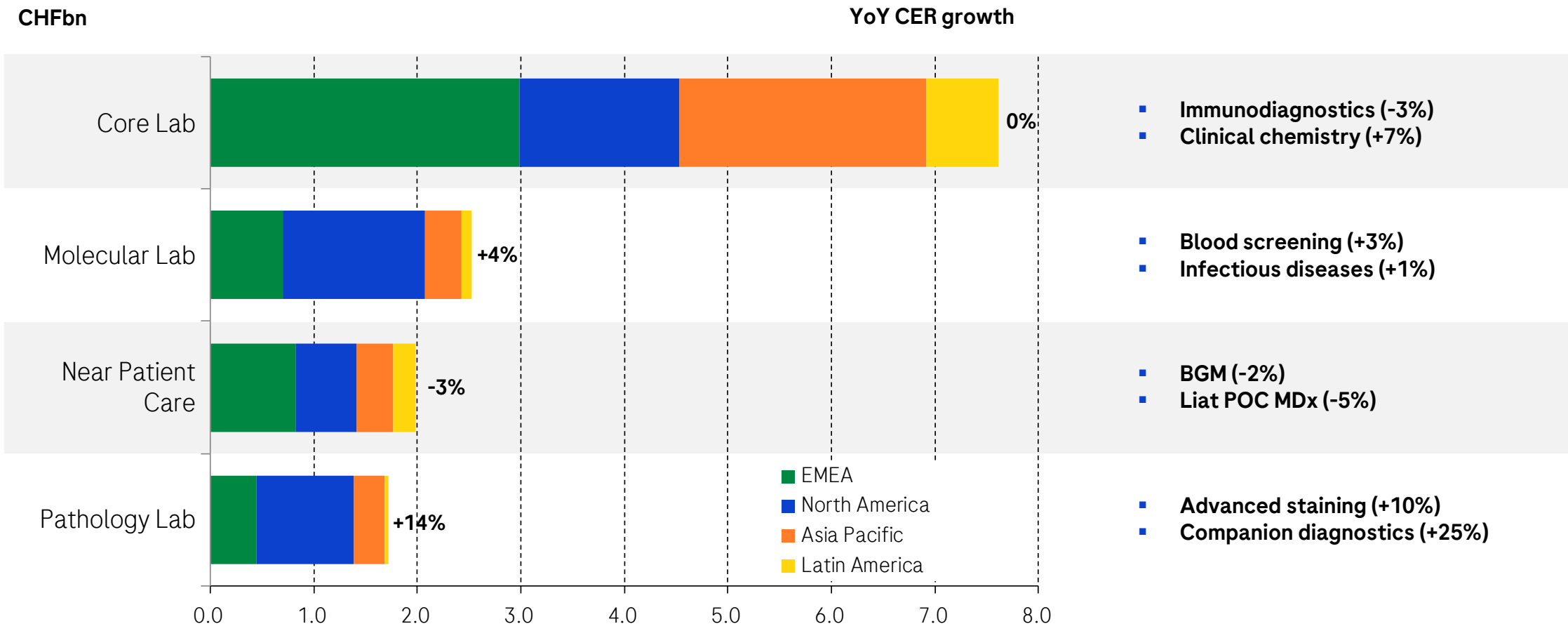
Diagnostics growing at +2%, impacted by China healthcare pricing reforms

	FY 2025 CHFm	FY 2024 CHFm	Change in % CHF	Change in % CER
Diagnostics Division	**13,847**	**14,324**	**-3**	**2**
Core Lab	7,614	8,011	-5	0
Molecular Lab	2,527	2,554	-1	4
Near Patient Care	1,983	2,160	-8	-3
Pathology Lab	1,723	1,599	8	14

CER: Constant exchange rates (avg. full year 2024)

2025: Diagnostics highlights

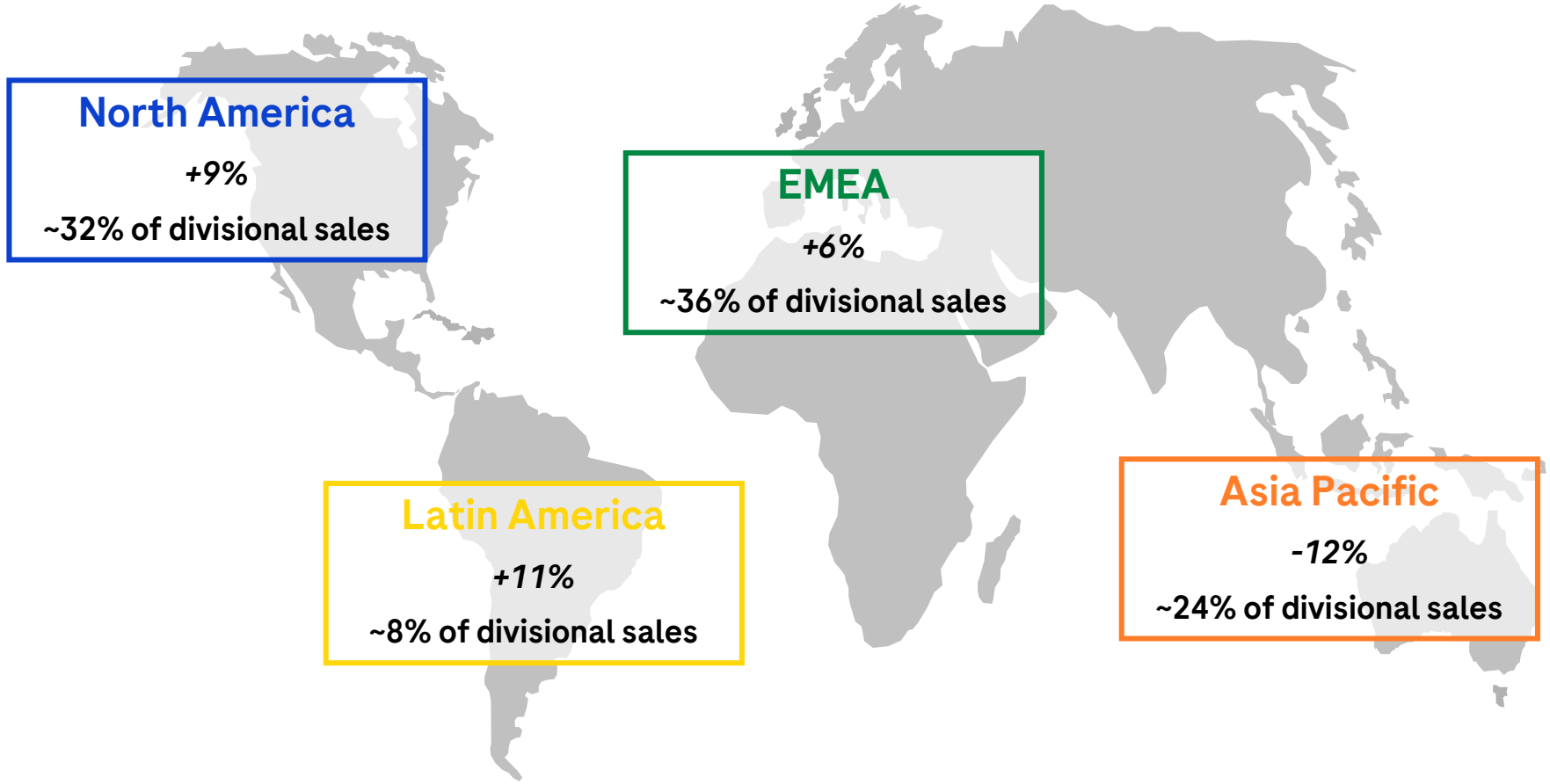
Diagnostics growing at +2%, impacted by China healthcare pricing reforms



BGM: Blood glucose monitoring; CER: Constant exchange rates (avg. full year 2024); EMEA: Europe, Middle East and Africa; POC MDx: Point of care molecular diagnostics.

2025: Diagnostics regional sales

Strong growth in Latin America, North America, and EMEA



Growth rates at constant exchange rates (avg. full year 2024); EMEA: Europe, Middle East, and Africa

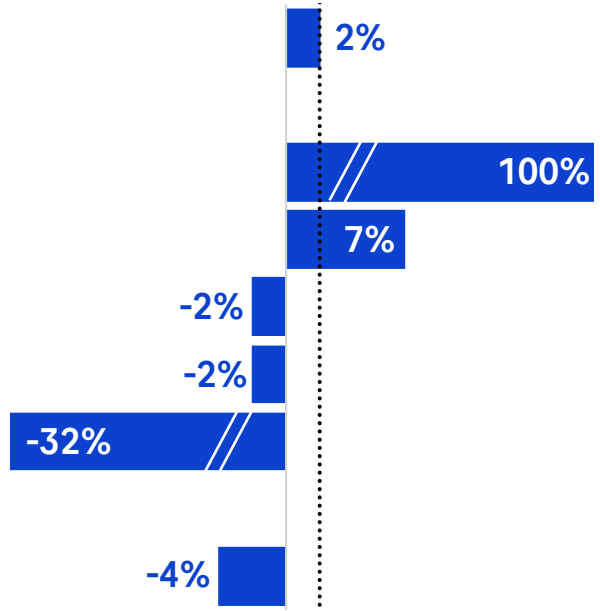
2025: Diagnostics Core operating profit

Core operating profit lower by -4% driven by China healthcare pricing reforms

	2025	
	CHFm	Abs. CERm
Sales	**13,847**	**+292**
Other revenue	58	+30
Cost of sales	-7,186	-512
R&D	-1,844	+39
SG&A	-2,923	+73
OOI&E	46	-23
Core operating profit	**1,998**	**-102**
Core OP as % of sales	*14.4%*	
At CER	*15.7%*	
	(2024: 16.8%)	

2025 vs. 2024

CER growth



-17% in CHF

CER: Constant exchange rates (avg. full year 2024); OOI&E: Other operating income & expense; OP: Operating profit; R&D: Research & development; SG&A: Selling, general & administration

cobas® i 601 Mass Spectrometry wave 1 menu CE mark

Mass spec assay menu expansion to cover the vast majority of routine testing demand

cobas® i 601 Mass Spectrometry wave 1 menu



Vitamin D

Steroid Hormones

Therapeutic Drug Monitoring *(ISDs, ABs, AEDs, 3MV)*

Market needs and opportunity

- Market Opportunity: The current clinical mass spectrometry market is CHF 3bn, composed of laboratory developed tests[1]
- Traditional mass spectrometry systems lack automation, are labour intensive and require high level of expertise

Differentiation

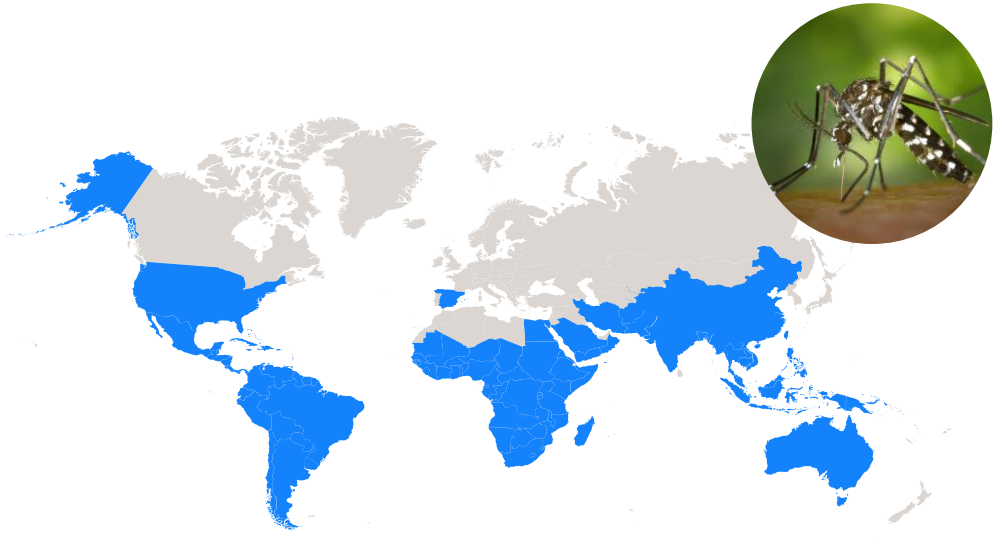
- Offer the broadest menu available for any automated mass spectrometry platform with 39 of the most common tests
- Provide an easy-to-use system and a broad menu for routine labs who perform TDM, Steroid Hormones and Vitamin D testing
- Replace labour-intensive manual workflows, reducing turnaround times and supporting IVD compliant and standardized testing

1. Roche commissioned external market research; 3MV: Mycophenolic acid, mycophenolic acid glucuronide, methotrexate, voriconazole (methotrexate available from 2026); AB: Antibiotics, AED: Antiepileptic drugs, ISD: Immunosuppressant drugs; IVD: In-vitro diagnostics; TDM: Therapeutic drug monitoring

Elecsys® Dengue Ag CE mark

Highly reliable automated serology assay for the detection of acute dengue infection

Elecsys® Dengue Ag test



>100 countries affected by dengue

Market needs and opportunity

- High Burden: Dengue is the most common mosquito-borne viral disease globally with an estimated 390m infections annually[1]
- Difficult Diagnosis: Patients are often misdiagnosed due to overlapping symptoms with other fever-causing illnesses[2]

Differentiation

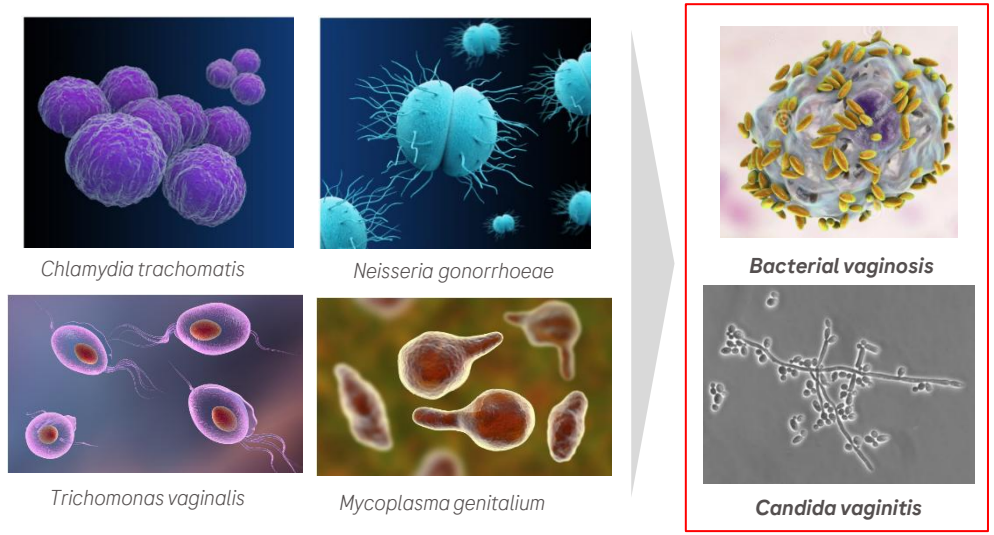
- Reliably detect all four dengue virus serotypes with a rapid 18-minute time-to-result test
- Distinguish dengue from other acute fever-causing illnesses, guiding appropriate patient management
- Leverage the fully-automated Elecsys installed base, supporting outbreak management and public health surveillance

1. The global distribution and burden of dengue, Bhatt et al. (2013), Nature; 2. PAHO-WHO. Dengue: guidelines for patient care in the Region of the Americas [internet; cited 2025 Jul 11]. Available from: https://iris.paho.org/handle/10665.2/31207; Chart depicts the map of countries affected by Dengue as per WHO Dengue and severe dengue (accessed in April 2025); Ag: Antigen

cobas® BV/CV CE mark

Important molecular menu gap fill to expand in the fast-growing molecular STI segment

Expanding STI menu



+ potential future expansion with at home collection (CT/NG, TV/MG, BV/CV, HPV) and point-of-care assays

Market opportunity

- The sexual health diagnostics market is valued at CHF 1.1bn with a yearly growth rate of 11%[1]
- Vaginitis is the primary growth driver in the STI segment, with a yearly growth rate of 26%[1]

Differentiation

- Provide a multiplex assay for the direct detection of bacterial vaginosis (BV) and candida vaginitis (CV)
- Enable testing of most common STIs including CT/NG, TV/MG, BV/CV by using a single test tube and vaginal swab
- Expand STI offering with future home collection solutions and new molecular point-of-care assays

1. FY24 Roche Market Book; BV/CV: Bacterial Vaginosis/Candida Vaginitis; CT/NG: Chlamydia trachomatis/Neisseria gonorrhoeae; HPV: Human Papillomavirus; STI: Sexually transmitted infections; TV/MG: Trichomonas vaginalis/Mycoplasma genitalium

cobas® liat Bordetella panel CE mark and FDA clearance

Rapid and accurate detection of Bordetella to enable early intervention and reduce potential complications

CE

FDA cleared

cobas® liat assay menu



- **cobas®** liat Bordetella panel
- **cobas®** liat CT/NG and CT/NG/MG
- **cobas®** liat SARS-CoV-2, Influenza A/B, RSV
- **cobas®** liat SARS-CoV-2 v2
- **cobas®** liat SARS-CoV-2 & Influenza A/B v2
- **cobas®** Influenza A/B & RSV
- **cobas®** Strep A
- **cobas®** C. diff

Market needs and opportunity

- High Burden: Pertussis causes 24.1 million cases annually, resulting in ~160,000 deaths/year often in young children[1,2]
- Difficult Diagnosis: Highly contagious pertussis is often underdiagnosed due to symptom overlap with common colds

Differentiation

- Detect and differentiate between three Bordetella pathogenic species with PCR-level accuracy
- Provide rapid and reliable results within 15 minutes enabling early intervention and reducing the risk of severe complications
- Expand Roche's menu of lab-equivalent molecular point of care solutions

Note: FDA Clearance with CLIA waiver; 1. CDC. Symptoms of whooping cough. [Internet; cited 2024 July]; 2.Yeung KHT, Duclos P, Nelson EAS, Hutubessy RCW. An update of the global burden of pertussis in children younger than 5 years: a modelling study. Lancet Infect Dis. 2017;17(9):974-980. doi:10.1016/S1473-3099(17)30390-0

Diagnostics key launches 2026

	Area	Product	Description	Market	Status
Systems	**Molecular Lab**	AXELIOS Sequencing Solution	Novel high-throughput solution for ultra-rapid, scalable, and flexible sequencing powered by Sequencing by Expansion (SBX) technology	Global	
Tests	**Core Lab**	Elecsys® pTau 217	Non-invasive blood-based biomarker to enable earlier detection, rule in and rule out Alzheimer's amyloid pathology	CE	
		Elecsys® NfL	Non-invasive blood-based biomarker to enable monitoring of disease activity in multiple sclerosis	CE	
		Elecsys® ApoE4p	Non-invasive blood-based biomarker to identify ApoE4 carrier, the strongest genetic risk factor for the development of Alzheimer's disease	CE	
		Elecsys® IGRA TB	Interferon-Gamma Release Assay (IGRA) to detect latent tuberculosis (TB) infection	CE	
	Molecular Lab	cobas® MPX-E	Molecular PCR test for the detection of HIV, HCV, HBV, and HEV in one assay for Donor Screening	CE	
		cobas® HDV	Molecular PCR test for aid in diagnosis of initial Hepatitis D virus infection and therapy monitoring	CE	
		eplex gastrointestinal panel	Multiplex Molecular PCR assay for the comprehensive detection of 19 potential pathogens associated with gastrointestinal infections	CE	
	Near Patient Care	cobas® liat lesion panel EUA	Rapid test to enable accurate detection and differential diagnosis of patients presenting with cutaneous and mucocutaneous lesions/ulcers to enable timely treatment and management. Supporting mpox health emergency	US EUA	
	Pathology Lab	VENTANA® BAP1	Highly specific marker for differentiating mesothelioma from benign mesothelial proliferation	CE	
Digital solutions	**Healthcare Insights**	LungFlag algorithm	Machine learning algorithm that identifies patients at high risk for lung cancer	CE	

ApoE4p: Apolipoprotein E4 protein; BAP1: BRCA1 Associated Protein-1; HCV: Hepatitis C Virus; HBV: Hepatitis B Virus; HDV: Hepatitis D Virus; HEV: Hepatitis E Virus; MPX-E: Multiplex HEV; NfL: Neurofilament light chain; PCR: Polymerase Chain Reaction; SBX: Sequencing by Expansion

Invitation to Diagnostics Day 2026



Roche Diagnostics Day: 12 May

London / hybrid event

14:00 - 16:45 CEST / 13:00 - 15:45 BST
08:00 - 10:45 am EDT / 05:00 - 07:45 am PDT

Doing now what patients need next

Roche Group development pipeline

Changes to the development pipeline

Q4 2025 update

New to phase I

5 NMEs:

RG6913 NME – DME
RG6496 HTT SNP ASO – Huntington's[1]
RG6498 NME - IBD
RGXXXX* B7H3 ADC – solid tumors[2]
RG6795 CDK4i - HR+ HER2- BC

New to phase II

1 AI:

RG6631 afimkibart (anti-TL1A) - rheumatoid arthritis

New to phase III

1 NME:

RG7935 prasinezumab - Parkinson's

1 AI:

RG6330 divarasib + pembrolizumab - 1L NSCLC

New to registration

1 NME (US):

RG6171 giredestrant + everolimus - post-CDK4/6 ER+/HER2- BC

1 AI (US):

RG7446 Tecentriq - ctDNA+ high-risk MIBC

Removed from phase I

3 NMEs:

RG6120 zifibancimig – nAMD
RG6221 LTBR agonist - solid tumors
RG6561 NME - solid tumors

Removed from phase II

1 NME:

RG6536 vixarelimab - IPF/SSc-ILD

Removed from phase III

1 AI:

RG7446 Tecentriq + platinum chemo – NSCLC periadj

Approvals

1 AI:

RG7828 Lunsumio SC - 3L+ FL

1 AI (EU):

RG7159 Gazyva - lupus nephritis

*RG Nr pending; [1]In collaboration with IONIS; [2]MediLink managed

Status as of January 29, 2026

Roche Group development pipeline

Phase I (40 NMEs + 6 AIs)

RG6026	Columvi monotherapy + combos	heme tumors
RG6076	englumafusp alfa combos	heme tumors
RG6160	cevostamab	r/r multiple myeloma
RG6171	giredestrant monotherapy + combos	solid tumors
RG6330	divarasib monotherapy + combos	solid tumors
RG6344	mosperafenib (BRAF inhibitor (3))	solid tumors
RG6411	enzelkitug (anti-CCR8)	solid tumors
RG6468	-	solid tumors
RG6505	PanRAS inhibitor	solid tumors
RG6537	AR degrader	mCRPC
RG6538	P-BCMA-ALLO1	r/r multiple myeloma
RG6540	P-CD19 x CD20 - ALLO1	heme tumors
RG6596	HER2 TKI	HER2+ BC
RG6620	KRAS G12D inhibitor	solid tumors
RG6648[1]	cMET ADC	solid tumors
RGXXXX*[1]	B7H3 ADC	solid tumors
RG7828	Lunsumio monotherapy + combos	heme tumors
RG6794	CDK4/2i	HR+ HER2- BC
RG6795	CDK4i	HR+ HER2- BC
RG6810[2]	DLL3 ADC	SCLC
RG6975[3]	CDH17 ADC	CRC
CHU	clesitamig (DLL3 trispecific)	solid tumors
CHU	codrituzumab	HCC
CHU	MINT91	solid tumors
CHU	anti-CTLA-4 switch antibody	solid tumors
CHU	pan-KRAS inhibitor (AUBE00)	solid tumors
RG6382	CD19 x CD3	SLE
RG6377	Anti-leukocyte trafficking antibody	IBD
RG6418**	selnoflast	inflammation
RG6421	TMEM16A potentiator	Muco-obstructive respiratory disease
RG6498	NME	IBD
RG6631	afimkibart (anti-TL1A)	MASH
RG7828	Lunsumio	SLE
CHU	anti-HLA-DQ2.5 x gluten peptides	celiac disease
CHU	anti-C1s recycling antibody	immunology
RG6035	Brainshuttle™ CD20	multiple sclerosis
RG6182	MAGL inhibitor	multiple sclerosis
RG6434	-	neurodegenerative disorders
RG6496[6]	HTT SNP ASO	Huntington's Disease
RG6662	HTT miRNA GT (SPK-10001)	Huntington's Disease
RG6209	VEGF x IL6 DutaFab	DME
RG6327	-	geographic atrophy
RG6913	-	DME
RG6006	zosurabalpin	bacterial infections
CHU	REVN24	acute diseases
CHU	BRY10	chronic diseases

Phase II (15 NMEs + 7 AIs)

RG6171	giredestrant	endometrial cancer
RG6180	autogene cevumeran	solid tumors
RG6797	SPK-8011QQ	hemophilia A
RG6512	NXT007	hemophilia
RG6287	flizasertib	CS-AKI
RG6631	afimkibart (anti-TL1A)	atopic dermatitis
RG6631	afimkibart (anti-TL1A)	rheumatoid arthritis
RG6730[4]	p40xTL1A bispecific antibody	UC
RG6237	emugrobart (GYM 329)	obesity
RG6641	GLP-1/GIP RA (CT-868)	T1D with BMI ≥ 25
RG6640	GLP-1/GIP RA (CT-388)	obesity +/- T2D
RG6849[5]	petrelintide	obesity +/-T2D
RG6652	GLP-1 RA (CT-996)	obesity +/- T2D
RG6042[6]	tominersen	Huntington's Disease
RG6168	Enspryng	DMD
RG6237	emugrobart (GYM 329) + Evrysdi	SMA
	emugrobart (GYM 329)	FSHD
RG6289	nivegacetor (gamma-secretase modulator)	Alzheimer's
RG6356[7]	Elevidys	0 to <4 year old DMD
RG7816	alogabat	Angelman syndrome
RG6351	Tie2 agonist	DME
RG6501	OpRegen	geographic atrophy

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology

Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

RG-No - Roche/Genentech; CHU - Chugai managed; [1]MediLink managed; [2]Innovent managed; [3]In collaboration with Hansoh Pharma, RG Nr TBD; [4]In collaboration with Pfizer; [5]Zealand Pharma managed; [6]In collaboration with IONIS; [7]US rights with Sarepta; *RG Nr pending; **also developed in neurology; T: Tecentriq; RA: Receptor agonist
CS-AKI: Cardiac surgery associated acute kidney injury

Status as of January 29, 2026

Roche Group development pipeline

Phase III (10 NMEs + 27 AIs)

RG3502	Kadcyla + T	HER-2+ eBC high-risk
RG6013	Hemlibra	Type 3 VWD
RG6026	Columvi + Polivy + R-CHP	1L DLBCL
	Columvi	r/r MCL
RG6107	PiaSky	aHUS
RG6114	Itovebi + fulvestrant	post CDKi HR+ PIK3CA-mut. BC
	Itovebi + Phesgo	1L HER2+ PIK3CA-mut. mBC
	Itovebi + CDK4/6i + letrozole	1L ES PIK3CA-mut. HR+ HER2- advanced BC
RG6171	giredestrant + palbociclib	1L ET sensitive ER+/HER2-mBC
	giredestrant	ER+ BC adj
	giredestrant + Phesgo	1L ER+/HER2+ BC
	giredestrant + CDK4/6i	1L ET resistant ER+/HER2- BC
RG6330	divarasib	2L NSCLC
RG6330	divarasib + pembrolizumab	1L NSCLC
RG7828	Lunsumio + lenalidomide	2L+ FL
	Lunsumio + Polivy	2L+ DLBCL
RG6149	astegolimab	COPD
RG6299[2]	sefaxersen (ASO factor B)	IgA nephropathy
RG6631	afimkibart (anti-TL1A)	ulcerative colitis
	afimkibart (anti-TL1A)	Crohn's disease
RG7159	Gazyva	membranous nephropathy
	Gazyva	systemic lupus erythematosus
	Gazyva	childhood onset idiopathic nephrotic syndrome*
RG6615[1]	zilebesiran	hypertension
RG6881	pegozafermin	MASH with fibrosis stage F2 or F3
	pegozafermin	MASH with fibrosis stage F4
RG6102	trontinemab	early Alzheimer's
RG6168	Enspryng	MOG-AD
	Enspryng	autoimmune encephalitis
RG6356[3]	Elevidys	amb. 8 to <18y & non amb. DMD
RG7845	fenebrutinib	RMS
	fenebrutinib	PPMS
RG7935	prasinezumab	Parkinson´s
RG6168	Enspryng	TED
RG6179	vamikibart	UME
RG6321	Susvimo	wAMD, 36-week
RG7716	Vabysmo	CNV

Registration US & EU (1 NME + 1AI)

RG6171	giredestrant + everolimus⁺	post-CDK4/6 ER+/HER2- BC
RG7446	Tecentriq	ctDNA+ high-risk MIBC

⁺Filed in US, FDA acceptance pending

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology
Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

Status as of January 29, 2026

[1]Alnylam Pharmaceuticals managed; [2]In collaboration with IONIS; [3]US rights with Sarepta; T:Tecentriq, *also known as pediatric nephrotic syndrome (PNS)

Expected regulatory submissions*

New Molecular Entities: Lead and additional indications

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology
Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

*Filing timelines reflect the anticipated filing of a potential indication; projects shown are in phase II and phase III
✓ Indicates submission to health authorities has occurred
Unless stated otherwise submissions are planned to occur in US and EU
T:Tecentriq, RA:Receptor agonist
**Filed in US, FDA acceptance pending; [1]Alnylam Pharmaceuticals managed

2025

RG6171	**giredestrant + everolimus ✓**** post-CDK4/6 ER+/HER2- BC
RG6321	**Susvimo ✓** wAMD (EU)

2026

RG6171	**giredestrant + palbociclib** 1L ET sensitive ER+/HER2- mBC
RG6171	**giredestrant** ER+ BC adj
RG7845	**fenebrutinib** RMS &PPMS
RG6179	**vamikibart** UME
RG6321	**Susvimo** DME (EU)

2027

RG6171	**giredestrant + Phesgo** 1L ER+/HER2+ BC
RG6171	**giredestrant + CDK4/6i** 1L ET resistant ER+/HER2- BC
RG6330	**divarasib** 2L NSCLC
RG6299	**sefaxersen (ASO factor B)** IgA nephropathy
RG6631	**afimkibart (anti-TL1A)** ulcerative colitis
RG6321	**Susvimo** wAMD, 36-week refill

2028 and beyond

RG6180	**autogene cevumeran** solid tumors
RG6330	**divarasib + pembrolizumab** 1L NSCLC
RG6512	**NXT007** hemophilia
RG6287	**flizasertib** CS-AKI
RG6631	**afimkibart (anti-TL1A)** Crohn's disease
RG6631	**afimkibart (anti-TL1A)** atopic dermatitis
RG6631	**afimkibart (anti-TL1A)** rheumatoid arthritis
RG6730	**p40xTL1A bispecific antibody** UC
RG6042	**tominersen** Huntington's
RG6102	**trontinemab** early Alzheimer's
RG6237	**emugrobart (GYM 329) + Evrysdi** SMA
RG6237	**emugrobart (GYM 329)** FSHD
RG6356	**Elevidys** 0 to <4 year old DMD
RG6356	**Elevidys** amb. 8 to <18y & non amb. DMD
RG7935	**prasinezumab** Parkinson's
RG7816	**alogabat** ASD
RG6501	**OpRegen** geographic atrophy
RG6351	**Tie2 agonist** DME
RG6237	**emugrobart (GYM 329)** obesity
RG6615[1]	**zilebesiran** hypertension
RG6640	**GLP-1/GIP RA (CT-388)** obesity +/- T2D
RG6641	**GLP-1/GIP RA (CT-868)** T1D with BMI ≥ 25
RG6652	**GLP-1 RA (CT-996)** obesity +/- T2D
RG6849	**petrelintide** obesity +/-T2D
RG6881	**pegozafermin** MASH with fibrosis stage F2 or F3
RG6881	**pegozafermin** MASH with fibrosis stage F4

Status as of January 29, 2026

Expected regulatory submissions*

Marketed products: Additional indications

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology

Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

✓ Indicates submission to health authorities has occurred
Unless stated otherwise submissions are planned to occur in US and EU
*Filing timelines reflect the anticipated filing of a potential indication; projects shown are in phase II and phase III
**also known as pediatric nephrotic syndrome (PNS)
***Filed in US

2025	
RG7446	**Tecentriq+ lurbinectedin ✓** 1l maintenance SCLC
RG7446	**Tecentriq ✓*** ctDNA+ high-risk MIBC

2026	
RG6107	**PiaSky** aHUS
RG6114	**Itovebi + fulvestrant** post CDKi HR+ PIK3CA-mut. BC
RG7828	**Lunsumio + lenalidomide** 2L FL+
RG7828	**Lunsumio + Polivy** 2L+ DLBCL (US)
RG7159	**Gazyva** membranous nephropathy
RG7159	**Gazyva** systemic lupus erythematosus
RG7159	**Gazyva** childhood onset idiopathic nephrotic syndrome**
RG6168	**Enspryng** MOG-AD
RG6168	**Enspryng** TED
RG7716	**Vabysmo** CNV

2027	
RG6013	**Hemlibra** Type 3 VWD
RG6114	**Itovebi + Phesgo** 1L HER2+ PIK3CA-mut. mBC
RG3502	**Kadcyla + Tecentriq** HER-2+ eBC high-risk
RG6026	**Columvi + Polivy + R-CHP** 1L DLBCL
RG6026	**Columvi** r/r MCL
RG6168	**Enspryng** autoimmune encephalitis

2028 and beyond	
RG6114	**Itovebi + CDK4/6i + letrozole** 1L ES PIK3CA-mut. HR+ HER2- advanced BC
RG6168	**Enspryng** DMD

Status as of January 29, 2026

Major pending approvals 2025

US		EU		China		Japan-Chugai	
RG6171	**giredestrant + everolimus** post-CDK4/6 ER+/HER2- BC Filed Dec 2025*	RG6321	**Susvimo** wAMD Filed July 2025	RG7596	**Polivy + chemo** r/r DLBCL Filed May 2025	RG7828	**Lunsumio + Polivy** 2L+ DLBCL Filed May 2025
RG7446	**Tecentriq** ctDNA+ high-risk MIBC Filed Nov 2025					RG7853	**Alecensa** ALK+ solid tumors Filed June 2025
						RG435	**Avastin** Neurofibromatosis Type 2 Filed August 2025
						RG7446	**Tecentriq** Adjuvant Therapy for MRD-Positive Bladder Cancer Filed January 2026

*FDA acceptance pending, ENKL : extranodal natural killer/T-cell lymphoma, nasal type

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology

Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

Status as of January 29, 2026

Major granted approvals 2025

US		EU		China		Japan-Chugai	
RG3625	**TNKase** stroke Feb 2025	RG6026	**Columvi + chemo** 2L DLBCL April 2025	RG7828	**Lunsumio** 3L+ FL Dec 2024	RG7446	**Tecentriq** Alveolar Soft Part Sarcoma Feb 2025
RG6321	**Susvimo** DME Feb 2025	RG6152	**Xofluza** influenza, pediatric (0-1 year) May 2025	RG6114	**Itovebi + palbociclib + fulvestrant** 1L HR+ PIK3CA-mut. mBC March 2025	RG6356	**Elevidys** DMD (ambulatory) May 2025
RG6321	**Susvimo** DR May 2025	RG6114	**Itovebi + palbociclib + fulvestrant** 1L HR+ PIK3CA-mut. mBC July 2025	RG1594	**Ocrevus** RMS & PPMS March 2025	RG7716	**Vabysmo** Angioid streaks May 2025
RG7446	**Tecentriq+ lurbinectedin** 1l maintenance SCLC Oct 2025	RG7828	**Lunsumio SC** 3L+FL Nov 2025	RG6026	**Columvi + chemo** 2L DLBCL April 2025	RG7446	**Tecentriq** ENKL Sept 2025
RG7159	**Gazyva** lupus nephritis Oct 2025	RG7159	**Gazyva** lupus nephritis Dec 2025			RG99	**CellCept** refractory nephrotic syndrome Sept 2025
RG7828	**Lunsumio SC** 3L+FL Dec 2025					RG7446	**Tecentriq** unresectable thymic carcinoma Dec 2025

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology
Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

Status as of January 29, 2026

Doing now what patients need next